OSCE COACH

The Ideal Answer Compendium

Every model answer from OSCE Coach, in one place

61 stations · **10** disciplines · ADC Practical Examination

Best read the night before, with a cup of tea
and absolutely no attempt to memorise it.

How to read this

This is every ideal answer from OSCE Coach, pulled straight from the station bank and grouped by discipline. It is the same text the app shows you after you finish a station — the comprehensive version, deliberately broader than anything you could say in ten minutes.

That last point matters, so here it is again in bold: **this is not a script.** No examiner expects you to deliver all of it. These answers exist to show you the full shape of the territory so that whichever corner of it the station lands in, you have already been there.

Read it for the patterns rather than the particulars. Notice how often the strong answer starts by acknowledging the person in front of you before reaching for a clinical fact. Notice how consistently a structured differential beats a confident guess. Notice how many stations are quietly testing whether you will do the safe thing when the patient is pushing you toward the fast thing. Those habits transfer between stations. The specific dose of amoxicillin does not.

> ***If you are reading this the night before:*** *skim the disciplines you find least comfortable, get some sleep, and trust the reps you have already put in. Cramming 60 model answers at midnight is not the move, and you already knew that.*

Sourced from the OSCE Coach station bank. Practice tool, not affiliated with the ADC.

Contents

PQ-4 Extraction Planning for a Patient on Denosumab and Warfarin

Oral surgery · 11 minutes · Cluster 2

The station. *Mr David Arthur is a 67 year old patient, attended your surgery today complaining of a swelling and tooth mobility in the lower right side. After examination, you have noticed that the lower right first molar has grade 3 mobility and abscess related to that tooth. The patient has a past medical history of Myocardial infarction, he also has Hypertension for which he is taking atenolol and warfarin. The patient also complains of hypothyroidism for which he is taking thyroxine, and osteoporosis for which he is taking calcium and denosumab iv injection only two days ago. He has also been taking panadeine forte. Manage patients concern and explain management.*

An excellent candidate would begin by warmly introducing themselves and establishing rapport with Mr David Arthur, acknowledging his discomfort and anxiety. They would take a thorough medication history, specifically asking about each medication by name — confirming that denosumab was given as a subcutaneous injection 60 mg only two days ago tabkets since 4 years, and asking about warfarin dose and INR (explains INR needs to be done within 24hours of the procedure, indication (in this case, history of MI with likely AF or thromboembolic risk), and most recent INR result.

The candidate would explain to Mr Arthur in plain language that the lower right first molar has a severe infection and is too mobile to save, and that extraction is the required treatment. They would then clearly explain the risk of medication-related osteonecrosis of the jaw (MRONJ) associated with denosumab — describing it as a condition where the jawbone struggles to heal after a tooth removal — and acknowledge that the very recent injection (two days ago) places the patient at elevated risk. They would explain that, unlike bisphosphonates, denosumab has a shorter half-life and its effects are more reversible, but the timing of extraction relative to the injection cycle still warrants specialist input.

Regarding warfarin, the candidate would correctly advise that warfarin does NOT need to be stopped for a simple extraction provided the INR is within the safe threshold (≤3.5–4.0), and that local haemostatic measures — including sutures, oxidised cellulose (Surgicel), and tranexamic acid mouthwash — will be used to control bleeding.

explaind MRONJ as exposed area of bone not healing for more that 8 weeks in patients taking anti resorptive drugs

The candidate would discuss liaising with the patient's GP or treating specialist before proceeding with definitive extraction, given the very recent denosumab dose. They would consider whether to temporise acutely — draining the abscess and prescribing antibiotics — while deferring the extraction to allow specialist consultation, rather than defaulting to immediate extraction. They would outline an atraumatic extraction technique, primary closure, antimicrobial cover, and close follow-up as part of an MRONJ prevention protocol. Informed consent covering both MRONJ and bleeding risks would be obtained and documented clearly.

PQ-6 Management of Crown Fracture During Extraction

Oral surgery · 11 minutes · Cluster 3

> ***The station.*** *Mr Parker 50 male, came to your surgery on Friday afternoon, he lives in two hours driving distance from the practice, with pain and swelling on upper molar. On examination 16/26 was badly decayed. You attempted extraction, the crown broke at gum level. Inform the pt about broken crown and discuss the next step of the treatment.pre operattive X-ray given showing molar roots are close proximity to sinus, pt taking blood pressure medicine, under control.*

An excellent candidate immediately stops the procedure, places gauze over the socket, and calmly informs the patient that the crown of the tooth has broken away during the extraction, which is a recognised complication, particularly with heavily decayed teeth. They explain in plain language that the roots are still present in the jaw and need to be removed, but that this requires a different, more careful approach. The candidate reassures the patient that she is still numb, that they will keep her comfortable, and that they are in control of the situation. They then review the radiograph, noting three divergent, bulbous roots in close proximity to the maxillary sinus floor, and examine the socket for visible root fragments, mobility, and any signs of oroantral communication — specifically asking whether the patient has noticed any air or fluid passing through, and inspecting the socket visually. Given the complex root anatomy, sinus proximity, and the risk of oroantral communication, the candidate explains that the safest next step is either a surgical approach involving a small gum flap and root sectioning if within their competence, or referral to an oral surgeon.

They discuss the realistic risk of an oroantral communication or root slipping into the sinus given the root apices' proximity to the sinus floor, and explain that if this occurs, management depends on the size: small communications can be managed conservatively with a suture and sinus precautions; larger ones require specialist repair root slipping management involves cad well luc approach by the specialist or leaving it there in less than 5 mm and not infected. The candidate addresses the patient's questions about pain (she will remain numb for now; post-operative analgesia such as ibuprofen and paracetamol will be prescribed), cost (private health insurance will likely contribute; they will provide an estimate), and timeframe. They document the complication in the clinical notes, obtain verbal informed consent for the next step, and provide written post-operative and sinus precaution instructions if proceeding, or a referral letter if referring.

appropriate follow up on call after 24 hours instruction and prescription

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PQ-13 Management of Post-Surgical Infection Following Extraction

Oral surgery · 11 minutes · Cluster 3

> ***The station.*** *Mrs Joshi is a regular patient at your clinic. You removed her upper left first molar (26) which was badly carious on Monday afternoon and today is Friday afternoon and she booked an emergency appointment, complaining about pain, swelling and bad breath which started 2 days after tooth was removed. After examining her mouth, you are suspecting it's a post-surgical infection. Manage patient's condition, explain the reason and further investigation that needs to be done. MH: Allergic to penicillin.*

An excellent candidate would begin by warmly greeting Mrs Joshi and acknowledging her discomfort before taking a focused history. They would establish the timeline: extraction on Monday, symptoms beginning Wednesday, now Friday — approximately 48 hours of progressive pain, swelling, and halitosis. They would ask about the nature and progression of swelling, any fever, difficulty breathing or swallowing, trismus, and what analgesia she has tried. They would take a thorough medical history and confirming allergy to penicillin. They would ask about her temperature, noting she recorded 37.8°C this morning.

On examination, the candidate would perform a thorough extra-oral assessment, noting the indurated, warm, erythematous swelling over the left mandibular body and submandibular region, measuring mouth opening at approximately 30mm indicating mild trismus, and critically checking for floor-of-mouth elevation, dysphagia, or any signs of airway compromise — all absent here. Intra-orally, they would inspect the socket for purulent discharge and assess surrounding tissues.

The candidate would diagnose a post-operative odontogenic infection and explain this clearly to Mrs Joshi in plain language, reassuring her while being honest about the need for prompt treatment. They would explain that diabetes impairs immune response and wound healing, increasing infection risk, and advise her to monitor blood glucose closely and contact her GP.

open disclosure accept that even though its a rare complication i should have informed you about it.

explain your management today(if temprature is more than 38 will require antibiotics) identify and remove the cause (if we are not able to identify the cause refer to hospital) xray for record and cinfirm if no rootpiece left behind

Given penicillin allergy, the candidate would prescribe cephalexin if allergy was mild or consider clindamycin 300mg TDS if allergy was severe. They would arrange review in 24–48 hours (if the swelling starts to improve continue treatment and if it doesnt improve refer to specialist suspecting oeteomyelitis) and provide clear safety-net advice: if she develops difficulty breathing or swallowing, floor-of-mouth swelling, rapidly worsening trismus, or high fever, she must present immediately to a hospital emergency department. Further investigations would include OPG to assess the socket and exclude retained root fragments.

PQ-14 Management of a Patient on Prednisone and Alendronate Requiring Extraction

Oral surgery · 11 minutes · Cluster 1

> ***The station.*** *Mrs Xiao is a 55-year-old patient attended your surgery today, complaining of badly decayed and she has been getting pain on 37 tooth for last 10 days. The tooth cannot be saved and needs extraction. Medically she is fit and well, she is taking Actonel tablets of 150mg once a month for osteoporosis condition for the last 4 years. She is also taking Prednisolone 10mg for her rheumatoid arthritis for the last 7 years. Please take consent for extraction and address patients concerns.*

An excellent candidate would begin by warmly greeting Mrs William, confirming her identity and the reason for her visit, then take a focused medication history. They would clarify that she is taking Actonel (risedronate) 150 mg monthly for osteoporosis for four years, and Prednisolone 10 mg daily for rheumatoid arthritis for seven years. They would note the concurrent use of a bisphosphonate and a corticosteroid as two independent risk modifiers for the planned extraction.

The candidate would then explain the risk of medication-related osteonecrosis of the jaw (MRONJ) in plain language: that bisphosphonates affect how bone heals after surgery,risk is increased by concurrent corticosteroid use and the duration of therapy exceeding four years. They would reassure Mrs Xiao that thats why it is preferred to do it with the specialist.

Next, the candidate would identify adrenal suppression risk. Seven years of Prednisolone 10 mg daily is sufficient to suppress the hypothalamic-pituitary-adrenal axis. Extraction under local anaesthesia is a moderate surgical stressor, and the candidate would discuss the need to liaise with her prescribing doctor regarding perioperative steroid supplementation.

For the management plan, the candidate would explain that the specialist would extract as indicated, with appropriate precautions: atraumatic surgical technique, primary wound closure where possible, chlorhexidine rinses, and consideration of prophylactic antibiotics.

Finally, they would provide smokinng cessation advise. prescription for pain. and review

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PQ-16 Management of Post-Operative Haematoma Following Dental Treatment

Oral surgery · 11 minutes · Cluster 2

The station. *Heather James is regular attendee at your clinic. booked an emergency appointment today complaining of swelling, mild discomfort and limited mouth opening following the IANB that was given a week before because of composite filling. You did examination today and you noticed patient can open up to 20mm, with swelling. You gave the diagnosis of trismus, hematoma and myospasm. Explain the diagnosis and how will you manage pts concern.*

An excellent candidate would begin by warmly greeting Mrs Hayes and acknowledging her concern about the alarming appearance of the swelling and bruising trismus. They would confirm any changes in medical history, confirmthe lenght and the difficulty of the procedure, and the timeline of swelling onset — noting it began within two hours and progressed overnight with bruising now tracking toward the neck.

They would specifically ask about medications, identifying aspirin 100mg daily and perindopril, and confirm she was correctly advised to continue aspirin. They would screen for red flag symptoms: any difficulty breathing, difficulty swallowing, worsening pain, fever, or rapidly increasing swelling — all of which are absent here. On examination, they would note extraoral ecchymosis and swelling, assess mouth opening nearly 20mm. make the patient understand why you gave a block anesthesia and not infiltration. The candidate would then explain the diagnosis clearly: the extraction caused minor bleeding from small blood vessels into the surrounding soft tissues, and because aspirin reduces platelet function, the blood spread more widely than usual, producing the bruising now visible on the face and neck. They would reassure Heather James that while the bruising looks frightening, it is not dangerous in her situation — it is self-limiting and will resolve over one to two weeks, changing colour from purple to yellow as it does.

Management advice for Hematoma would include warm compresses now to aid reabsorption 20 minutes / hour, continuing regular paracetamol for mild discomfort,

management for trismus maintaining a soft diet, and keeping the area clean.

The candidate would safety-net clearly: Mrs James should return immediately or go to an emergency department if she develops any difficulty breathing or swallowing, rapidly increasing swelling, fever above 38°C, or signs of infection such as pus or worsening pain.

They would offer a review appointment in five to seven days to assess Hematoma and Trismus if improving continue with the treatment if hematoma is not improving refer to oral surgeon (may need MRI to assess where the blood is, risk of infection and scarring) if trismus is not improving refer to oral medicine specialist to assess for TMD

PQ-19 torres islander nurse

Oral surgery · 11 minutes · Cluster 3

The station. *Torre islander nurse comes today complaining of pain from 16. You did the examination and 16 has irreversible pulpitis and insisted on pulling it out. Now you have decided to extract the tooth. Xray shows that 16 roots are close to the sinus and there is an infection around 1 root. Patient is worried about OAC, Hepatitis B and HIV infection as she heard of a breach in some other dental clinic. manage patients concerns.*

An excellent candidate would begin by introducing themselves, demonstrating cultural respect from the outset. They would take a focused medical history, absence of allergies, and regular GP contact at the Aboriginal Medical Service. explains the xray The candidate would then assess the severity of the dental infection by asking about fever, swelling progression, systemic symptoms, recognising that while the infection is localised, it requires urgent management. They would explain clearly that tooth 16 infected and needs to remove the source of infection

Demonstrates knowledge of current Australian guidelines: Standard protocol set by DBA to prevent HIV and Hep B infection spread usually they spread by sharps injury during procedures. Sharps management(to prevent injuries) rusable instruments are transferred safely in lidded tray sterlized as per their category.non critcal and critical. Critical instrument protocol includes washed / ultrasonic bath/ pouched/ BCI/ chemical indicators inside every pouch in every cycle / goes into patients notes / opened at point of use to ensure safety.

understanding of Australian system treatment free for Torres Islanders in community clinics and in private clinic treatment is chargable but they are provided vouchers and specialist treatment available at a discount.

The candidate would address the proximity of roots to the sinus, explaining the risk of oro-antral communication and how they would manage this if it occurred.

management prescrib pain medicines (Antibiotic needed if it is apical periodontitis and you are not treating today) short term tooth removal/RCT/temporisation(pulpectomy) long term replacement options

use health promotion to consider why everyone in family extracts teeth (maybe they dint have access to treatment)

PQ-25 Consent for Extraction

Oral surgery · 11 minutes · Cluster 3

The station. *Tooth 16 is badly broken down and unrestorable — it needs extraction. Explain the procedure, describe the complications (common and serious), give post-operative instructions, and confirm the patient's understanding before proceeding. X-ray provided showing tooth 16 (upper right first molar) grossly carious and unrestorable, with the roots lying in close proximity to the floor of the maxillary sinus. Smoking history.*

The candidate begins by confirming the patient's identity and the tooth to be extracted, clearly stating 'tooth 16, the upper right first molar.' They acknowledge Mrs Collins' anxiety and reassure her that her comfort is the priority throughout. The candidate explains the procedure in plain language: local anaesthetic will be given to numb the area, the tooth and surrounding gum will be gently loosened, and the tooth will be removed using dental instruments. They note the procedure typically takes 15–30 minutes. The candidate explains why extraction is recommended — the tooth is too badly broken to restore — and briefly mentions that root canal treatment was considered but is not viable given the extent of the damage. Common risks are discussed clearly: some pain and swelling for a few days, possible bruising, minor bleeding, difficulty opening the mouth, and a small risk of damage to adjacent teeth. Serious risks specific to this tooth are addressed: dry socket (alveolar osteitis), oro-antral communication given the proximity of the upper molar roots to the maxillary sinus, possible root fracture requiring surgical retrieval, and that nerve injury is rare for this tooth. The candidate addresses Mrs Collins' smoking history directly, advising her to avoid smoking for at least 48–72 hours as it significantly increases the risk of dry socket. Post-operative instructions include biting on gauze for 30 minutes, avoiding rinsing for 24 hours, taking paracetamol and/or ibuprofen as directed, eating soft foods, and returning if bleeding does not settle or pain worsens after day three. The candidate confirms she can drive home as local anaesthetic alone does not impair driving, and advises she should be able to return to work the following day in most cases. The candidate checks Mrs Collins' understanding, invites questions, and confirms her voluntary consent before documenting in the clinical notes.

PQ-32 Non-Healing Extraction Socket

Oral surgery · 11 minutes · Cluster 1

The station. *55 year old female patient had extraction done 4 days ago by another dentist across the road and pain started soon afterwards. Pt complains of bad smell and bad taste. Give possible causes of pain, what investigations will you do and manage patients concern.*

An excellent candidate would begin by introducing themselves and establishing rapport with Mrs Chen, acknowledging her discomfort and the fact that she has been in pain for several days. They would take a focused history covering the timeline of the extraction, the duration and difficulty of the procedure, the onset and character of the pain, any bad taste or smell, and what analgesia she has already tried. They would specifically ask about her medical history, confirming her type 2 diabetes and asking about recent blood sugar control and HbA1c, her antihypertensive medication, and importantly screen for bisphosphonate or anticoagulant use. Smoking history would be quantified in pack-years. On examination, the candidate would inspect the socket at site 46, noting the exposed greyish bone, absence of a blood clot, minimal granulation tissue, halitosis, and tenderness on gentle probing, while confirming the absence of swelling, lymphadenopathy, purulent discharge, or paraesthesia. The provisional diagnosis of dry socket (alveolar osteitis) would be clearly stated, with acknowledgement that poor glycaemic control (HbA1c 8.2%) and active smoking are significant complicating factors that impair healing. The candidate would explain the condition to Mrs Chen in plain language — that the protective blood clot has been lost from the socket, leaving the bone exposed and causing the severe pain and odour. Immediate management would include gentle irrigation of the socket with warm saline, placement of an obtundent dressing such as Alvogyl, and upgrading analgesia to ibuprofen 400mg with paracetamol 500–1000mg alternating four-hourly if not contraindicated, or referral for prescription analgesia if inadequate. The candidate would advise Mrs Chen to see her GP urgently for HbA1c review and optimisation of diabetic control, provide brief smoking cessation advice and offer referral to Quitline (13 7848), and arrange review in 48–72 hours. Safety-netting would include clear instructions to return immediately if she develops increasing swelling, fever, trismus, or difficulty swallowing.

PQ-42 Post-Operative Instructions for Patient on Liquid Diet

Oral surgery · 11 minutes · Cluster 3

The station. *David Chen, 28-year-old office administrator. Scenario: You have just surgically removed both lower wisdom teeth (38, 48) — raised mucoperiosteal flaps, bone removal with handpiece, sectioning of teeth, and 3-0 Vicryl sutures placed bilaterally. No complications during surgery. Local anaesthetic given (articaine with 1:100,000 adrenaline); patient is still numb. Medical/Social History: No medical conditions, no regular medications. Non-smoker. Lives alone in Parramatta, works full-time in an office, has taken 2 days off work. First time having oral surgery, mildly anxious. Clinical Image: No intraoral clinical photo — text-only slide on dark background. Task: Give post-extraction…*

An excellent candidate would begin by confirming what was done and checking the patient's understanding, acknowledging that both lower wisdom teeth were surgically removed today and that the sites are still numb from the local anaesthetic. They would explain that because raised flaps, bone removal, and sutures were involved, the healing sites need protection, which is why a liquid diet is recommended for the first 24 to 48 hours, progressing to soft foods over the following five to seven days. The candidate would give specific examples of suitable foods — cool or lukewarm soups, smoothies, protein shakes, milk, and yoghurt drinks — while advising the patient to avoid hot, cold, or spicy foods that could irritate the sites. They would clearly advise against using straws, explaining that the suction pressure can dislodge the blood clot and lead to dry socket, a painful complication. For general post-operative care, the candidate would advise the patient to avoid rinsing for the first 24 hours to protect the clot, then begin gentle warm salt water rinses from the following day. They would recommend applying ice packs to the cheeks for the first 24 hours to manage swelling. Regarding analgesia, the candidate would confirm that paracetamol 1g every six hours is available at home and would recommend adding ibuprofen 400mg every eight hours with food, taken in alternation, explaining this combination is more effective than either alone. They would advise the patient that two days off work is appropriate given the bilateral surgical nature of the procedure. Safety-net advice would include warning signs requiring urgent review: excessive or uncontrolled bleeding, severe worsening pain after day two or three suggesting dry socket, fever, swelling that is increasing rather than decreasing, or any difficulty swallowing or breathing. The candidate would provide the practice's emergency contact number and confirm the patient has a way to get home safely given they are still numb.

PQ-49 Management of Apical Root Fragment Retained During Extraction

Oral surgery · 11 minutes · Cluster 3

The station. *50+ year-old male. Came to the surgery on a Friday afternoon. Lives 2 hours driving distance from the practice. Scenario: Patient presented with pain and swelling on upper right molar side which was badly decayed. Was advised for extraction and agreed. During extraction, palatal root fractured and small apical 1/3 is left behind. Discovered that tooth had a vertical crack after extraction. On radiograph, root was in close proximity to maxillary sinus. Patient is taking blood pressure medication. Currently under control. Clinical Image: Periapical X-ray of the upper molar region showing retained root tip/fragment with a highlighted circle indicating its location close to the maxillary…*

An excellent candidate would begin by pausing the procedure, ensuring the patient is comfortable, and calmly disclosing what has occurred. They would explain in plain language that during the extraction a small piece of the root tip — approximately 3 to 4 millimetres — has broken away and remains in the bone near the upper right back tooth area. They would acknowledge the patient's likely concern and normalise the situation by explaining this is a recognised complication of difficult extractions, particularly when teeth have curved or fragile roots or underlying cracks, as was found here. The candidate would then explain the clinical findings: the fragment is small, it is sitting close to the floor of the maxillary sinus, there is no sign of active infection, and the socket does not appear to be communicating with the sinus. They would present two management options clearly. First, attempting immediate surgical retrieval, which carries risks including excessive bone removal, potential perforation of the maxillary sinus, prolonged procedure time, and increased post-operative discomfort. Second, a conservative monitoring approach, which is supported by evidence for small, asymptomatic apical fragments in the absence of infection or sinus communication — such fragments often become encapsulated or resorb without causing problems. Given the fragment size, absence of infection, proximity to the sinus, and the patient's well-controlled blood pressure, the candidate would recommend conservative management with close follow-up. They would involve the patient in this decision, answer her questions honestly, and obtain verbal consent for the monitoring plan. Clear post-operative instructions would be provided: avoid nose-blowing, watch for increasing pain, swelling, discharge, or a bad taste, and return immediately if any of these occur. A review appointment would be arranged in one to two weeks with a follow-up periapical radiograph at three to six months. The incident would be documented fully in the clinical notes including the fragment size, location, radiographic findings, discussion held, and the agreed management plan.

PQ-55 Oroantral Communication — Disclosure After Upper Molar Extraction

Oral surgery · 11 minutes · Cluster 3

The station. *Mrs Elida Jones, regular patient. Scenario: You performed an extraction of tooth 26 (upper left first molar). During the extraction, the tooth fractured. You successfully removed approximately 2/3 of the tooth but 1/3 of the root (~1–2 mm) remains in the socket. On testing sinus patency (Valsalva / nose-blow test), air is bubbling from the socket — confirming oroantral communication (OAC). Task: Explain to the patient what has happened during the extraction. Inform her about: (1) the retained root fragment, (2) the oroantral communication, and (3) immediate and ongoing management. Address her concerns.*

An excellent candidate opens by gently orienting Mrs Jones before delivering the news, signalling that there is something important to discuss. They disclose both complications — the retained root fragment and the oroantral communication — clearly and without delay, using plain language throughout. They explain that the tooth fractured during removal, which is a recognised risk with heavily restored upper molar teeth, and that while most of the tooth was successfully removed, a small root tip of approximately one to two millimetres remains. They explain that attempting to retrieve this fragment now carries a greater risk of pushing it into the sinus than leaving it under specialist review, and that a specialist will assess it within the next day or two. They then explain the oroantral communication in lay terms — that a small opening has formed between the mouth and the sinus cavity above, confirmed by the bubbling seen when she breathed out through her nose. When Mrs Jones asks 'Did you do something wrong?', the candidate responds honestly: the tooth fractured, this is a known complication particularly with heavily restored upper molars, and while they are genuinely sorry it happened, it does not represent a failure in technique. The candidate asks about penicillin allergy before prescribing amoxicillin 500 mg three times daily for five days, and adds a nasal decongestant spray. They ask about prior sinus problems, elicit the history of resolved chronic sinusitis, and explain why this makes careful follow-up especially important. They give specific OAC precautions — no nose blowing for two weeks, sneezing with the mouth open, no straws, no smoking. They arrange urgent referral to an oral and maxillofacial surgeon within 24 to 48 hours. When Mrs Jones raises concern about living alone, they provide written instructions, a practice after-hours number, and clear criteria for seeking emergency care.

Periodontics

11 stations

PQ-1 Orthodontic Patient with periodontal problem

Periodontics · 11 minutes · Cluster 1

The station. *Jordan 17-year-old boy came to your practice for consultation for crowded teeth. He has seen a dentist 12 months ago. You did CPITN scoring today (Community Periodontal Index of Treatment Needs) which is 444/424 and you have taken bitewing x-ray showing bone loss around molars. Intraoral picture showing crowding and deep bite on anterior. Consult with patient regarding findings.*

An excellent candidate opens by acknowledging Jordan's expectation of a quick referral visit, then pivots to explain that the examination has revealed something important that needs to be addressed first. The candidate explains in plain language that the gum measurements — called CPITN scores — show pocketing of 6 mm or more in all back teeth, and that the X-rays confirm bone loss around the molars. The candidate clearly states that this level of disease in a 17-year-old is not normal and requires investigation beyond a routine scale and clean. The candidate then takes a targeted history: asking specifically about brushing frequency, flossing, and toothbrush type; asking directly and non-judgmentally about cigarette or vape use; asking whether any family members have had gum problems or lost teeth early; and asking specifically whether anyone in the family has diabetes or other systemic conditions. When Jordan discloses his father's early tooth loss and grandmother's type 2 diabetes, the candidate connects these to a possible familial predisposition to aggressive periodontitis and a potential systemic component. The candidate also asks about stress, sleep, and diet, connecting HSC pressure to immune function and periodontal risk. The candidate articulates a differential that includes Stage III Grade C periodontitis, periodontitis as a manifestation of systemic disease. Management is explained clearly: immediate referral to a periodontist for specialist diagnosis and management; GP referral for fasting glucose and HbA1c given the family history of type 2 diabetes; specific oral hygiene instruction including twice-daily brushing and daily flossing; a brief non-judgmental smoking cessation message; and an explanation that orthodontic treatment must be deferred until the gums are stable, because moving teeth through inflamed bone accelerates bone loss and risks tooth loss. The candidate recommends 3-monthly review and explains why this is more frequent than a standard check-up.

PQ-11 Generalised Periodontitis With Replacement Options

Periodontics · 11 minutes · Cluster 2

The station. *Miss Luiz is a 55 yr old patient, attending your clinic today complaining of pain, bleeding gums and mobility of left lateral incisor (22). OPG given with boneless around most of the teeth and lateral has boneless up to 90%. Perio chart provided with probing depth of 7-8mm around some of the teeth. MH: she is taking thyroxine for hypothyroidism, bone pain and coeliac disease and taking calcium, vitamin D for bone. Patient wanted to know about replacement option.*

An excellent candidate would begin by warmly greeting Miss Luiz and confirming her main concerns — pain, bleeding gums, and the wobbly front tooth. They would explain, in plain language, that she has a condition called generalised periodontitis, where bacteria under the gumline have caused the bone that holds her teeth in place to dissolve away over many years. Using the OPG and perio chart, they would show her that most teeth have lost significant bone support, and that the upper left lateral incisor (tooth 22) has lost approximately 90% of its bone, making it non-restorable and requiring extraction. They would acknowledge her medical history — hypothyroidism managed with thyroxine, coeliac disease, and supplementation with calcium and vitamin D — noting these are relevant to healing and bone quality.

. They would outline a staged treatment plan: first, initial periodontal therapy including full-mouth debridement and thorough oral hygiene instruction; second, extraction of hopeless teeth including tooth 22; third, a healing and reassessment phase of at least 8–12 weeks, GP referral for blood tests and bone scan. short term options immediate denture and fiber reinforced bridge

discussion of definitive replacement options.

For replacemen long termt, they would present various options — a removable partial denture (most affordable, non-invasive, but less comfortableput pressure on bone and aesthetic) maryland being long term preferred option, a dental implant (most functional and aesthetic long-term solution, but requires adequate bone costs she already has lots of bone loss and poor bone quality so less preferred option for her) approximately $4,000–6,000 per implant in Australia, with a longer timeline), and no replacement for posterior teeth with an explanation of the drifting and occlusal consequences. They would involve the patient in shared decision-making, address her concerns about appearance and cost, and arrange a follow-up appointment for periodontal therapy.

PQ-17 Chronic Generalised Periodontitis Assessment and Management

Periodontics · 11 minutes · Cluster 2

> ***The station.*** *Mr Boldoski is a 55 yrs old patient, attending your clinic today complaining of bleeding gums and wanted some cleaning. You have examined the patient. OPG taken today and you can see generalized bone loss. You gave diagnosis as generalised periodontitis, stage 4 and grade C. His medical records show that he didn't see GP for 10 yrs. You decided to refer him to specialist. Explain the reason for referral and address pts concern.*

An excellent candidate would begin by warmly greeting Mr Boldoski and acknowledging his concern about bleeding gums and his desire for cleaning. They would take a focused periodontal history, asking about the duration of bleeding, any pain, tooth mobility, previous dental visits, and current oral hygiene practices. They would then systematically explore medical and social history, specifically asking about smoking and diabetes, and would elicit that Mr Boldoski smokes 10 cigarettes per day with a 30-year history

also linking underlying medical conditions (diabetes/hormonal/ nutritional deficiency) as the patient hasnt done any GP visits in 10 years.

The candidate would explain the diagnosis in plain language: that Mr Boldoski has a serious gum infection called Stage 4 Grade C periodontitis, where bacteria have caused significant bone loss around the teeth — up to 40-50% in some areas — and that somel teeth already show mobility. They would explain that this is not simply a matter of 'needing a clean' but a complex condition requiring specialist-level care.

The candidate would clearly explain the reason for specialist referral: the severity of bone loss, multiple deep pockets up to 8mm, tooth mobility on four teeth, vertical bony defects, and the compounding risk factors of uncontrolled diabetes and heavy smoking, which accelerate disease progression and impair healing. They would explain that a periodontist can offer advanced non-surgical and surgical options, including full-mouth debridement, possible osseous surgery, and long-term maintenance planning.

the candidate would strongly encourage Mr Boldoski to see his GP to review his blood work for diabetes nutritional deficiency

Smoking cessation would be addressed empathetically but directly, with a referral to Quitline (13 7848) offered. The candidate would address cost concerns by explaining that early specialist treatment is more cost-effective than eventual tooth loss and implants. They would check understanding throughout and reassure Mr Boldoski that with compliance, tooth loss can be minimised.

if sensitivity is missed in the history then highly likely its management is not addressed (acute care)(not eating due to sensitivity can affect blood sugar level)

PQ-22 Acute Periodontal Abscess

Periodontics · 11 minutes · Cluster 2

The station. *Jane Highland 55 came to your clinic with sore 11 and 21 with mobility grade II. extruded 11 with 7mm probing depth. generalised 4 to 5 mm probing depth. The patient is fit and healthy, but she smokes 25 cigarettes/day. Manage the case, give the diagnosis, and explain treatment options.*

I would begin by introducing myself and establishing rapport before taking a focused history. I would ask Mrs Jane HIghlands about the onset, duration, and severity of her pain, noting it started two days ago and worsened yesterday, is throbbing in nature, rates 8/10, and is associated with a bad taste suggesting purulent discharge. I would ask about swelling, fever, difficulty swallowing, and malaise.

Crucially, I would ask about her medical history, specifically her diabetes Communicates the link between diabetes control and periodontal disease; advises patient to see GP for diabetes review and emphasises importance of compliance with periodontal maintenance and GP visit to review Diabetes status and any deficiencies (explains that unmanaged gum disease can affect glycemic control

perform vitality testing to confirm the tooth is vital — a critical finding that differentiates a periodontal abscess from a periapical abscess. I would probe the pocket, finding a 7 mm pocket with purulent discharge 11 21. The radiograph confirms 60% bone loss with a vertical defect distally and no periapical pathology.

My primary diagnosis is an acute periodontal abscess on tooth 11 21 with moderate generalised chronic periodontitis Stage III Grade B as the underlying condition. The differential includes periapical abscess, which is excluded by confirmed vitality and absence of periapical radiolucency.

Immediate management involves drainage via the pocket using a curette, irrigation with saline, and if fluctuant, incision and drainage. I would advise continuing paracetamol and ibuprofen.

explainds that the diastema and extrusion could self correct after craining and managing gum condition first (and explains further management after review in 1 month if they dont improve)

I would review her in 2 days to assess resolution,on review appointmentdiscuss managemnt of extruded tooth(might go back after we drain the abscess/ trim / refer for ortho) and disatema (goes back after drainage/ filling / ortho) and then schedule generalized periodontal management with specialist therapy including full-mouth debridement under local anaesthesia once the acute phase resolves. I would counsel Mrs Jane Highland on the bidirectional relationship between diabetes and periodontal disease, strongly advise a GP review for diabetes management, and reinforce the importance of 3-monthly periodontal maintenance and daily interdental cleaning and smoking cessation

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PQ-37 Management of Peri-Implantitis

Periodontics · 11 minutes · Cluster 3

The station. *Peri-implantitis affecting an implant-supported bridge spanning 32 to 42. A radiograph and clinical picture are provided showing peri-implant marginal bone loss of approximately 4 mm (beyond expected crestal remodelling), with peri-implant redness, bleeding on probing, and probing depths of 6–7 mm. The patient is a smoker; the main concern is the bleeding gums. Explain the diagnosis, treatment options, and management, and answer the patient's questions.*

An excellent candidate would begin by introducing themselves and establishing rapport with Mr Chen, acknowledging his concern about bleeding gums and anxiety about losing his implant. They would take a focused history asking about the duration of symptoms, noting redness for 6–8 months and mild chewing discomfort for 2 months, and would directly ask about smoking, eliciting the 10 cigarettes per day history. They would confirm oral hygiene habits, brushing frequency, flossing, and recall attendance.

The candidate would describe or perform a thorough clinical examination including probing depths at six sites per implant, recording 7–8 mm depths with bleeding on probing and suppuration, assessing implant mobility (absent), and noting visible plaque accumulation. They would request or review a periapical radiograph confirming approximately 4 mm of bone loss beyond expected crestal remodelling, meeting the threshold for peri-implantitis diagnosis.

The candidate would explain the diagnosis clearly: peri-implantitis is an infection around the implant causing bone loss, more serious than early gum inflammation around implants (peri-implant mucositis), and is the reason for the bleeding and discomfort. They would distinguish this from mucositis in plain language.

Risk factors would be addressed directly and sensitively: smoking significantly impairs healing and increases bone loss risk, once-daily brushing without interdental cleaning allows plaque to accumulate around implant components, and irregular recall has allowed the condition to progress undetected.

Management would be outlined in stages: non-surgical debridement using implant-safe instruments (carbon fibre or titanium curettes, ultrasonic with plastic tips), oral hygiene instruction including interdental brushes, smoking cessation referral or NRT discussion, and possible adjunctive local or systemic antimicrobials such as chlorhexidine gel or metronidazole if indicated. The candidate would explain that if non-surgical treatment fails to resolve the infection and halt bone loss, surgical intervention may be required, and in refractory cases implant removal may be necessary. A review appointment at 8–12 weeks would be arranged, with emphasis on regular 3-monthly maintenance thereafter.

PQ-45 Management of Acute Necrotising Ulcerative Gingivitis

Periodontics · 11 minutes · Cluster 2

The station. *John Smith, 25-year-old male. Chief Complaint: 'My teeth are bleeding and they hurt. It stings and makes it hard to eat.' Problem has occurred on and off for the past year, gotten worse over the past two months. Went to GP this morning and was given penicillin but hasn't bought it yet. Examination Findings: Ulcers visible around the margins of the gum. Medical/Social History: Medically fit and healthy. Had a viral infection 10 days ago. Smokes 10 cigarettes/day. Takes alcohol once a day. Law student with upcoming exam — stated it was quite stressful. Allergy to penicillin. Clinical Image: Frontal intraoral photo showing classic NUG/ANUG presentation. Task: Explain diagnosis and…*

An excellent candidate would begin by introducing themselves and establishing rapport before taking a focused history. They would clarify the onset, duration, and character of pain, ask about recent stress, smoking, diet, sleep, and any systemic symptoms such as fever or lymphadenopathy. Crucially, they would identify the penicillin allergy documented in the brief before any antibiotic discussion. On examination, they would note the pathognomonic punched-out interdental papillae with grey-yellow pseudomembrane, spontaneous gingival bleeding, severe halitosis, and generalised marginal erythema, while confirming the patient is afebrile and systemically well with no lymphadenopathy. They would confidently diagnose Acute Necrotising Ulcerative Gingivitis, explaining to the patient in plain language that this is a bacterial gum infection triggered by the combination of stress, smoking, poor sleep, and reduced oral hygiene. They would reassure the patient that ANUG is not contagious. Immediate management would include gentle supragingival debridement with warm saline or dilute hydrogen peroxide irrigation, oral hygiene instruction using a soft brush, and advice to avoid smoking. Because the patient has a penicillin allergy, they would prescribe metronidazole 400 mg twice daily (BD) for three to five days rather than amoxicillin, with clear instructions to take with food and avoid alcohol. Appropriate analgesia such as ibuprofen 400 mg with food or paracetamol 1 g if NSAIDs are contraindicated would be recommended. The candidate would advise adequate hydration, nutritious soft foods, and improved sleep. They would arrange review in 24 to 48 hours to assess response, and explain that once the acute phase resolves, definitive periodontal treatment including scaling and root planing will be required. Smoking cessation support would be offered with referral to Quitline 13 7848.

PQ-46 Chronic Generalised Periodontitis Assessment and Management

Periodontics · 11 minutes · Cluster 2

The station. *Mrs Wilson, 45-year-old, new patient. Presentation: Attending today requesting a scale and clean — previous dentist of 20 years has recently retired. Medical history clear. Smokes 10 cigarettes/day. Family history of early tooth loss. Has brought along an OPG taken at her last dental appointment (tooth 48 extracted). Examination Findings: Full periodontal examination performed including periodontal charting. Heavy plaque and calculus deposits, with some bleeding on probing at deeper sites. Generalised probing depths of 5–6 mm. Recession of up to 2 mm on the lower anterior teeth. Clinical Image: OPG showing generalised alveolar bone loss, with extraction site at tooth 48. Task: Explain…*

An excellent candidate would begin by warmly welcoming Mrs Wilson and acknowledging that changing dentists after 20 years can feel unsettling. They would take a focused periodontal history, confirming the timeline of any symptoms such as bleeding gums, sensitivity, or loose teeth, and asking directly about her smoking habit and daily oral hygiene routine. After reviewing the OPG and periodontal chart, the candidate would explain the diagnosis in clear lay terms: that Mrs Wilson has a serious form of gum disease called generalised periodontitis, where the bone and supporting structures around most of her teeth have been significantly damaged, and that this goes well beyond what a routine scale and clean can address. The candidate would sensitively link her smoking — ten cigarettes per day — and her family history of early tooth loss as important risk factors that both worsen the disease and reduce the body's ability to heal. They would outline a staged treatment plan: beginning with thorough oral hygiene instruction and full-mouth scaling and root planing under local anaesthesia, followed by a reassessment at six to eight weeks to evaluate the tissue response. They would explain that some teeth may require further specialist periodontal treatment or, if they cannot be saved, extraction. The candidate would strongly emphasise smoking cessation as the single most impactful change Mrs Wilson can make, offering referral to Quitline (13 7848) or her GP for pharmacotherapy support. Prognosis would be discussed honestly but with hope: with compliance, regular three-monthly supportive periodontal therapy, and smoking cessation, disease progression can be halted and teeth retained. The candidate would acknowledge Mrs Wilson's concerns about cost and time, explaining that treatment can be staged to manage both, and that investing in periodontal control now is far less costly than tooth replacement later. Throughout, the candidate would seek Mrs Wilson's understanding and consent at each stage.

PQ-47 Management of Chronic Generalised Periodontitis

Periodontics · 11 minutes · Cluster 2

The station. *Mrs Wilson, 45-year-old, new patient. Presentation: Attending today requesting a scale and clean — previous dentist of 20 years has recently retired. Medical history clear. Smokes 10 cigarettes/day. Family history of early tooth loss. Has brought along an OPG taken at her last dental appointment (tooth 48 extracted). Examination Findings: Full periodontal examination performed including periodontal charting. Heavy plaque and calculus deposits, with some bleeding on probing at deeper sites. Generalised probing depths of 5–6 mm. Recession of up to 2 mm on the lower anterior teeth. Clinical Image: OPG showing generalised alveolar bone loss, with extraction site at tooth 48. Task: Explain…*

An excellent candidate would begin by warmly introducing themselves and establishing rapport with Mrs Wilson, acknowledging that changing dentists after 20 years can feel unsettling. Before explaining the diagnosis, the candidate would elicit a focused periodontal history — asking about bleeding gums, tooth mobility, previous gum treatment, and current oral hygiene practices including brushing frequency, flossing, and use of interdental aids. Crucially, the candidate would specifically ask about smoking history in pack-years and probe for any systemic conditions, including diabetes, recognising that the examiner context reveals these are hidden risk factors the patient will only disclose if directly asked.

Having gathered this information, the candidate would explain the diagnosis of chronic generalised periodontitis in plain language — describing it as a serious but treatable infection of the supporting structures around the teeth, where bacteria under the gumline have caused the bone and gum to pull away, creating pockets of 5–6 mm. They would reference the OPG to show the generalised bone loss visually and explain that the recession on the lower front teeth is a visible sign of this process.

The candidate would then clearly link smoking and diabetes to worsened disease severity and poorer healing, using non-judgmental, motivational interviewing language to explore readiness to quit smoking and encourage liaison with her GP regarding diabetes control and the bidirectional periodontal-systemic relationship. They would recommend a structured management plan: personalised oral hygiene instruction, full-mouth non-surgical periodontal therapy (scaling and root planing) delivered across multiple appointments, a 6–8 week reassessment, and referral to a periodontist if inadequate response. Cost and time concerns would be addressed honestly, explaining the staged approach and the long-term cost of inaction — progressive tooth loss mirroring her family history. Informed consent would be documented, along with a plan to write to her GP.

PQ-51 Perio Patient — Replacement Options for Missing Teeth (OPG, Bone Loss)

Periodontics · 11 minutes · Cluster 1

> ***The station.*** *45-year-old male, new patient. Has been seeing another dentist. Presenting for a second opinion regarding missing teeth and replacement options. Examination Findings: OPG provided showing generalised bone loss and multiple missing posterior teeth. Patient is known to have significant bone loss. Has not been formally diagnosed at this visit. Task: Gather information. Advise on investigations required. Give appropriate replacement options for missing teeth. No need to provide a formal diagnosis. Focus on patient communication, exploring options (implants, bridges, partial dentures), and realistic expectations given bone loss.*

An excellent candidate would open by establishing rapport with David in a direct, no-nonsense manner, acknowledging that he is here for a second opinion and that he deserves straight answers. The candidate would take a focused history covering: duration and frequency of dental attendance, any prior periodontal treatment, compliance with recall, oral hygiene habits including the cessation of flossing, smoking history (ten cigarettes per day for twenty years — a 10 pack-year history), alcohol use, and medical history including a specific question about diabetes. On identifying that David stopped flossing because his gums bled, the candidate would gently but clearly correct this misconception, explaining that bleeding gums are a sign of inflammation and that stopping interdental cleaning allows the disease to progress unchecked. The candidate would then explain the OPG in plain language: the bone that anchors the teeth has reduced by roughly half in most areas as a result of longstanding gum disease, and several posterior teeth are already missing. The candidate would be honest that this is significant but not hopeless. When discussing replacement options, the candidate would cover all three modalities — implants, fixed bridges, and removable partial dentures — with honest pros and cons. Critically, the candidate would explain that none of these options can be pursued until the active gum disease is brought under control first. Regarding implants specifically, the candidate would confirm that implants are a realistic goal but would explain that smoking approximately doubles the risk of implant failure, that smoking cessation is strongly recommended before proceeding, and that bone grafting may be required in areas of significant resorption. Fixed bridges would be noted as potentially unsuitable given that the neighbouring teeth are themselves periodontally compromised. Removable partial dentures would be presented as the most immediately available option but the least preferred long-term. The candidate would close by reassuring David that options remain available, that this is not a hopeless situation, but that the sequence matters: stabilise the gum disease, re-evaluate, then plan replacements.

PQ-56 Drug-Induced Gingival Enlargement — Cyclosporine + Nifedipine

Periodontics · 11 minutes · Cluster 3

The station. *Mr Richard Jones, regular attender. Chief Complaint: Swollen and bleeding gums. Has a wedding to attend next week — wants it resolved quickly. Examination Findings: Generalised gingival enlargement noted on examination. Periodontal charting performed. Medical History: Kidney transplant — taking cyclosporine for 15 years (immunosuppressant); Hypertension — taking nifedipine (calcium channel blocker) for the past 8 months; Taking atorvastatin for cholesterol. Drug-Induced Gingival Overgrowth: Both cyclosporine and nifedipine are well-known causes. The combination significantly increases risk and severity. Task: Diagnose and manage. Address the patient's concerns. Discuss the drug-gingival…*

An excellent candidate would begin by warmly acknowledging Mr Jones's concern about the wedding and signalling that they will address it directly. Through targeted history-taking, the candidate would establish the precise timeline: gums were stable for 15 years on cyclosporine alone, and the swelling and bleeding began approximately 8 months ago — correlating exactly with the introduction of nifedipine. The candidate would then deliver a clear diagnosis: drug-induced gingival overgrowth, caused by the combination of cyclosporine and nifedipine, both of which independently cause this condition, and together produce a significantly worse effect. The mechanism would be explained in plain language — these medications alter the behaviour of cells in the gum tissue, causing them to produce too much fibrous tissue, and plaque and bacteria make this considerably worse. The candidate would explicitly advise Mr Jones not to stop or alter either medication himself, explaining that stopping nifedipine could cause a dangerous rise in blood pressure, and stopping cyclosporine could risk rejection of his transplanted kidney — both decisions belong with his medical team. The management plan would follow a clear ladder: immediate intensive oral hygiene instruction and full-mouth scaling and root planing, with a review at six to eight weeks to assess tissue response. The candidate would commit to writing a liaison letter to the GP explaining the diagnosis, identifying nifedipine as the likely precipitating agent given the timeline, and requesting consideration of substitution with an antihypertensive with a lower association with gingival overgrowth, such as an ACE inhibitor or ARB. The transplant physician would also be contacted regarding the cyclosporine component and a potential drug interaction. Regarding the wedding, the candidate would be honest: professional cleaning will reduce bleeding and acute inflammation and should produce some visible improvement within a week, but meaningful structural resolution of the overgrowth itself is unlikely in that timeframe and may take several months.

NEW Ortho Referral — Mild Periodontitis (PQ-1 Variation)

Periodontics · 11 minutes · Cluster 1

The station. *Maya Chen, a 16-year-old Year 11 student, has come to your practice wanting a referral for braces to fix her crowded teeth — her cousin's wedding is in four months and she'd like a plan in place. Last dental visit was 12 months ago. You have done CPITN scoring today (Community Periodontal Index of Treatment Needs), which is 3, 3, 3 / 3, 2, 3. Bitewing X-rays show no significant bone loss. Intraoral picture shows moderate crowding of the lower anteriors with generalised mild gingival inflammation and visible calculus on the lower incisors. Consult with the patient regarding the findings.*

This is the same referral request as PQ-1 — a teenager with crowding who wants braces — but a different periodontal picture, and the correct answer is different too. The candidate who treats every elevated CPITN score in a teenager as a red flag will over-manage this case; the candidate who ignores it will under-manage it. The skill being tested is reading the severity correctly.

READING THE SCORE

CPITN 3, 3, 3 / 3, 2, 3 means five sextants have pockets of 4-5mm, and the lower anterior sextant scores 2 — calculus present, but no true pocketing. That lower-anterior finding is common and unremarkable on its own; that sextant sits opposite the salivary duct openings and accumulates calculus fastest of anywhere in the mouth. This is a world away from PQ-1's 4, 4, 4 / 4, 2, 4 — pockets of 6mm or more in every posterior sextant, which is genuinely abnormal for this age and demands a specialist work-up. Here, a code of 3 still means the affected sextants need detailed periodontal charting today — with five of six elevated, that is effectively full-mouth charting — but code 3 disease is managed with oral hygiene instruction and non-surgical periodontal therapy, not an automatic periodontist referral.

OPENING

Maya expects a quick referral conversation. Acknowledge that before redirecting: "I can see you're keen to get moving on braces, and we will get there. Before I write the referral, I want to talk you through what I found when I checked your gums today, because it changes the plan a little — in a good way, I think."

THE HISTORY

Even though this case turns out to be straightforward, take the full history — that is what separates a candidate who got the right answer by luck from one who reasoned their way there:

- Oral hygiene: brushing frequency, technique, and specifically whether she flosses. She uses an electric toothbrush twice a day but has never flossed regularly — she tried once and the floss kept shredding on her crowded lower front teeth, so she gave up. That detail matters: it tells you the crowding is actively obstructing her ability to clean, which is the mechanism behind the findings.
- Family dental and medical history, including diabetes — worth asking on every case like this, not because you expect a positive answer, but because a negative one is genuinely useful information. Here it comes back clean: no early tooth loss in the family, no diabetes. Treat that as evidence against a familial or systemic cause and let it narrow your thinking, rather than continuing to hunt for a hidden systemic driver that isn't there.
- Smoking or vaping — no. Stress, sleep, diet — nothing concerning; Year 11 is a calmer year than the HSC pressure a Year 12 student might be under.
- Previous dental advice — her last dentist, a year ago, mentioned her gums "could be better" and suggested a clean she never booked.

THE DIAGNOSIS

Put it together: generalised mild gingival inflammation with early, localised periodontal breakdown, driven by inconsistent interdental cleaning that her crowding makes genuinely difficult — not Stage III/Grade C aggressive periodontitis, and not periodontitis as a manifestation of systemic disease. The bitewings support this: no significant bone loss, consistent with early disease rather than established, longstanding attachment loss.

MANAGEMENT

- Full periodontal charting today.
- Oral hygiene instruction tailored to her actual problem — floss keeps shredding, so move her to interdental brushes sized for her tighter contacts, or a water flosser, alongside her existing electric toothbrush technique.
- Non-surgical periodontal therapy — scaling and root surface debridement. This sits comfortably within general practice; there is no need for a periodontist here.
- Review in 6-8 weeks to reassess bleeding on probing and pocket depths.
- Once that review shows improvement — which is the expected outcome — proceed with the orthodontic referral. Frame this as a sequence, not a refusal: "Once your gums have settled down, which I'd expect in a couple of months, we'll get that referral off. And honestly, straightening these crowded teeth will make it much easier for you to keep them clean long-term, so this actually sets the orthodontic treatment up

better."

- If, despite good compliance, there's no improvement at review, that's the trigger to refer to a periodontist — stating this escalation pathway shows judgement, not just a rehearsed answer.
- The one thing that doesn't change from the more severe case: orthodontic forces should not be applied to an actively inflamed periodontium, because it risks further attachment loss. The difference is that here, that wait is a matter of weeks with treatment you can deliver yourself — not an indefinite hold pending specialist stabilisation.

WHY THIS MATTERS

The wedding in four months gives Maya a real but non-clinical deadline. Being honest that a few weeks of perio treatment now still gets her to the orthodontist in plenty of time turns a conversation she might experience as bad news into one she leaves feeling good about.

COMMON WAYS TO FAIL THIS STATION

Treating CPITN 3 the same as CPITN 4 and referring straight to a periodontist. Never mentioning the perio findings and just writing the ortho referral she asked for. Failing to ask why she doesn't floss and missing the crowding-hygiene link entirely. Continuing to probe for a systemic cause after the family history comes back clean. Giving no review interval or escalation pathway. Leaving her with the impression that braces are off the table rather than delayed by weeks.

Endodontics

5 stations

PQ-9 RCT patient wants Extraction

Endodontics · 11 minutes · Cluster 1

The station. *Mr Argenti is attending your clinic today and complaining of severe pain coming from one of the tooth on the top left side. You have taken the opg and you can see that some teeth have been Root canal treated few months ago. Patient is very angry and would like to remove these teeth. Give your differentials and explain the necessary investigations and explain extraction option.*

This station looks like an endodontic case and is not one. The pain is non-odontogenic: Mr Argenti has maxillary sinusitis driven by hay fever he has never treated. Everything the station rewards flows from the candidate resisting the pull toward the tooth.

OPENING

Acknowledge before you assess. Something like: "Mr Argenti, I can hear how frustrating this is — three months of pain, and treatment that hasn't fixed it. I do want to help. Before I can take a tooth out, I need to be sure which tooth is causing this, otherwise I could take one out and leave you in exactly the same pain. Can we work through it together?" That framing does two jobs at once — it validates him, and it pre-justifies the questions he is about to resist.

THE QUESTION THE CASE TURNS ON

"Is this pain the same as the pain you had before the root canals, or is it different?" This is the discriminator. Unchanged pain, untouched by definitive endodontic treatment, points away from the tooth. Pain that is new or different in character since the treatment points toward a failed RCT or a vertical root fracture. Here it is the same as before — it never changed — which should immediately widen the differential beyond the teeth.

DRAWING OUT THE HIDDEN HISTORY

He will not volunteer the sinus picture. Ask specifically:

- What makes it worse? (Bending forward — putting shoes on, leaning over the sink. Classic sinus sign.)
- Any blocked nose, stuffiness, congestion, drip at the back of the throat?
- Any bad breath or bad taste? (He has halitosis and is embarrassed about it.)
- Any allergies or hay fever? (Yes — for years. Taking nothing for it. This is the root of the case.)
- Smoking? (Ex-smoker.) Alcohol? (Regular.)
- Fever, feeling generally unwell?

DIFFERENTIAL DIAGNOSIS

Odontogenic: failed or incomplete root canal treatment — a missed canal or persistent bacteria; vertical root fracture. Non-odontogenic: maxillary sinusitis; respiratory infection or allergic rhinitis (hay fever). Two pieces of reasoning worth saying out loud: root canal treatment has a high success rate, so several treated teeth in one quadrant all still hurting should itself make you suspicious of a non-dental cause. Conversely, repeated unilateral sinusitis should raise suspicion of a sinusitis of odontogenic origin — the maxillary posterior root apices sit right against the sinus floor.

INVESTIGATIONS TODAY

- Extra-oral: swelling, fever, palpate over the maxilla and the jaw. (Tender over the left cheek; no swelling, no fever.)
- Intra-oral: examine every restoration and the gingivae. (Sound; no sinus tract.)

- Percussion and palpation. (Diffusely tender across several upper left teeth — no single localised offender. That non-localisation is itself the finding.)
- Cold/vitality testing. Important nuance: the root-filled teeth will not respond, and that is expected, not pathology. Test the adjacent non-treated teeth — they respond normally.
- Probing — specifically looking for the narrow isolated defect of a vertical root fracture. (Normal depths; argues against VRF.)
- Bite test / tooth slooth for a crack. (Negative.)
- Radiographs: periapicals plus the OPG. Root fillings are adequate with no clear periapical radiolucency, but the left maxillary sinus is cloudy with mucosal thickening — fluid in the sinus. Cloudiness on an OPG is easy to skip past and is the radiographic key here. CBCT via the specialist would confirm.
- SELECTIVE ANAESTHESIA — the decisive test. "Can I make that tooth numb for you? It'll help the pain, and it'll also tell us whether this is coming from the tooth or from the sinus." If the pain fully resolves, the source is that tooth. Here it only partially reduces — which tells you the source is the surrounding structures, not the tooth.

WHAT TO SAY ABOUT THE EXTRACTION

"I'm genuinely happy to take a tooth out for you — but only if we can localise it. At the moment several teeth are tender rather than one, and when we numbed that tooth the pain only partly settled. That tells me the pain isn't coming from the tooth itself. If I take it out today, I'd be taking out a healthy tooth and you'd still have the pain." Respect his autonomy, but do not extract. This is the line the station is testing.

MANAGEMENT

Refer to an endodontist, who will work with ENT to establish the cause — that joint pathway is the expected answer. GP referral for the hay fever and the sinusitis; antibiotics are a GP decision if he develops fever or becomes systemically unwell. Provide interim analgesia advice and book a definite review — he must not leave with nothing after three months of pain.

HEALTH PROMOTION

The untreated hay fever is the single biggest opportunity in this station and the one most candidates miss — he has had it for years and takes nothing for it, and it is plausibly driving the whole picture. Direct him to his GP or pharmacist for management. Reinforce staying off cigarettes (he has already quit — congratulate rather than lecture). Discuss alcohol intake, which also feeds the halitosis. Explain the causes of bad breath, including that a sinus or ear infection can cause it — that will reassure him. Arrange regular dental review.

THE COMPLAINT ABOUT THE PREVIOUS DENTIST

He will ask whether the last dentist did a bad job, and how to complain. Handle it in three parts:

- Stay neutral and factual. Do not criticise the colleague and do not speculate about negligence — you were not there, you have not seen their records, and on the evidence so far the endodontic treatment looks adequate. Say what you can see, not what you assume. "From what I can see the root fillings look well done. I can't comment on another dentist's treatment beyond what's in front of me."
- Do not dismiss him either. He is entitled to complain and it is not your place to talk him out of it.
- Give the actual pathway: raise it directly with the treating dentist or the practice first — most concerns are resolved at that level. If he wants to take it further, a formal complaint about a registered practitioner goes to Ahpra and the Dental Board of Australia; in New South Wales it goes to the Health Care Complaints Commission, and in Queensland to the Office of the Health Ombudsman. State health complaints bodies also handle service-level complaints. He is entitled to a copy of his records, and you can help him request them.

COMMON WAYS TO FAIL THIS STATION

Agreeing to extract. Never asking whether the pain is the same as before. Taking "no response to cold" on a root-filled tooth as a positive finding. Never asking about the nose. Missing the hay fever entirely, or finding it and doing nothing with it. Criticising the previous dentist. Refusing to engage with the complaint. Ordering investigations without being able to say what each one would tell you.

PQ-23 Management of Acute Irreversible Pulpitis in a Patient Flying Interstate

Endodontics · 11 minutes · Cluster 2

The station. *Mr John Adam is attending your clinic today as an emergency appointment for broken tooth on Lower right back (46). On examination, badly broken tooth, Patient going away for 2 days interstate, no gingival or apical swelling, no lymphadenitis, explain diagnosis, emergency, and definitive management. Pain management.*

An excellent candidate would begin by warmly greeting Mr Adam and establishing rapport before taking a focused pain history. They would ask about the onset, character, duration, and progression of pain — eliciting that it began three days ago as a cold-triggered ache but has evolved into severe spontaneous throbbing pain that wakes him at night. They would ask about aggravating and relieving factors, noting that lying down worsens pain and that cold water previously offered some relief but no longer does. They would screen for systemic symptoms including facial swelling, fever, and malaise to rule out spreading infection. A brief medical history would confirm no allergies, no relevant medications, and no contraindications to NSAIDs.

The candidate would then explain the diagnosis clearly: the nerve inside the tooth is severely inflamed and cannot heal on its own — this is called irreversible pulpitis. They would specifically warn Mr Adam about barodontalgia, explaining that pressure changes during the 2.5-hour flight can dramatically worsen dental pain, and that flying with untreated irreversible pulpitis carries a real risk of severe, unmanageable pain mid-flight.

For emergency management, the candidate would strongly recommend performing a pulpectomy — removing the inflamed nerve tissue — today before travel, as this is the most effective way to eliminate the pain source. They would explain the procedure briefly and reassuringly. If the patient declines or time does not permit, the candidate would offer to arrange an emergency dental referral in Brisbane, providing contact details for emergency dental services there.

For analgesia, the candidate would prescribe a combination of ibuprofen 400mg every six to eight hours with food and paracetamol 1g every six hours, staggered for continuous coverage, explaining that medication alone is unlikely to fully control irreversible pulpitis pain. They would advise Mr Adam to take analgesia before boarding, stay hydrated, and contact emergency dental services in Brisbane if pain escalates. The candidate would acknowledge his anxiety and conference commitments with genuine empathy throughout.

PQ-24 Post-Endodontic Pain Management Following Recent Root Canal Treatment

Endodontics · 11 minutes · Cluster 3

The station. *Patient had RCT done on lower right back tooth 3 days ago with another dentist. Previous dentist didn't explain complications of RCT. Patient has an appointment with a dentist in 2 weeks. She came to see you today because she has pain after the treatment. On examination, band (stainless steel) was given to tooth and GIC filled (temporary restoration). Pain on biting, not sensitive to hot or cold. X-ray provided shows slightly inadequate obturation. Patient wants pain management and wants to know why previous dentist did RCT when there was a crack, anything wrong with RCT.*

An excellent candidate would begin by warmly introducing themselves and establishing rapport, acknowledging that the patient is in pain and that this situation must be distressing. They would take a focused pain history, establishing that the pain is 6–7 out of 10, throbbing, constant, and worse on biting, and that paracetamol and ibuprofen together have provided only minimal relief. They would ask about the timeline — that the crack was identified two weeks ago after biting hard bread, RCT was performed three days ago, and a temporary restoration with a stainless steel band and GIC was placed. They would screen for signs of infection by asking about fever, facial swelling, discharge from the gum, and trismus, and would note the absence of these features. Medical history would be taken, identifying well-controlled hypertension managed with perindopril, and confirming no drug allergies. On examination, the candidate would check the temporary restoration for integrity, perform percussion testing (noting marked tenderness on 46), assess mobility (none), palpate the buccal mucosa (slightly tender, no swelling or sinus), check for lymphadenopathy (absent), inspect for the crack line, and critically check the occlusion — identifying a high spot on the temporary restoration. A periapical radiograph would be requested and interpreted, noting obturation to working length in all three canals, a widened PDL space apically, and a faint crack line. The provisional diagnosis would be post-endodontic pain that is likely multifactorial: occlusal trauma from the high temporary restoration, normal post-operative periapical inflammation, and possible ongoing crack propagation. Management would include immediate occlusal adjustment of the temporary restoration, optimisation of analgesia with alternating paracetamol and ibuprofen on a scheduled basis, monitoring for developing infection, and clear communication that a definitive crown is urgently needed to splint the cracked tooth and prevent further propagation. The candidate would explain the rationale for RCT in a cracked tooth in plain language and address the patient's concerns empathetically.

PQ-41 Diagnosis and Management of Vertical Root Fracture

Endodontics · 11 minutes · Cluster 2

The station. *Mr John Hobbart, existing patient. Chief Complaint: Dull ache in a lower back tooth for a few weeks — the tooth had root canal treatment about three years ago. Examination Findings: Tooth 36 — large MOD amalgam restoration, mild tenderness to percussion, no mobility. Slight swelling and erythema of the buccal gingiva adjacent to the tooth. Medical History: Well-controlled hypertension (perindopril 5mg daily). No known drug allergies. Non-smoker, social drinker. Clinical Image: Periapical radiograph available on request. Task: Take a history, examine as needed, formulate a diagnosis, and explain your management plan to the patient.*

An excellent candidate would begin by introducing themselves and establishing rapport with Mr Hobbart before taking a focused history. They would ask about the onset, duration, and character of the pain, any previous dental treatment on the tooth, and specifically whether there has been any trauma. They would elicit the history of root canal treatment completed three years ago, the intermittent gingival swelling that comes and goes, and the patient's avoidance of chewing on the left side. The candidate would then perform a systematic clinical examination including percussion, palpation, mobility testing, and careful periodontal probing of the affected tooth and all adjacent teeth, noting the isolated narrow deep pockets of 6–7 mm on the mesial and distal aspects of tooth 36 against normal 2–3 mm depths on adjacent teeth. They would request a periapical radiograph and correctly identify the J-shaped radiolucency along the mesial root surface with loss of lamina dura, interpreting this alongside the clinical findings. The candidate would formulate a diagnosis of vertical root fracture, explaining that the combination of a root-canal-treated tooth, isolated deep narrow pockets, intermittent sinus tract, and J-shaped radiolucency is pathognomonic for this condition. They would communicate clearly and empathetically to Mr Hobbart that the tooth cannot be saved and requires extraction. They would then discuss replacement options including an osseointegrated implant, a three-unit fixed dental prosthesis, or a removable partial denture, as well as the option of no replacement, outlining the pros, cons, and approximate costs of each. Before proceeding, the candidate would note the patient's medical history — well-controlled hypertension on perindopril — and confirm there are no contraindications to extraction or implant placement. They would arrange a follow-up appointment, provide written information, and check the patient's understanding throughout.

PQ-48 Consent for Endodontic Treatment

Endodontics · 11 minutes · Cluster 3

The station. *Mr Jonathan, 27-year-old, regular patient. Scenario: You saw him in the last visit and gave the diagnosis of irreversible pulpitis for tooth 16. Patient is back again complaining about pain and does not want extraction. Medical History: Medically fit and healthy. Doesn't drink or smoke. Clinical Image: Periapical radiograph showing tooth 16 (upper first molar) with roots in close proximity to the maxillary sinus. Task: Explain the procedure and gain informed consent for endodontic treatment (roots close to sinus).*

The candidate begins by confirming the patient's identity and the tooth requiring treatment, clarifying that today's appointment is to discuss root canal treatment for tooth 16, the upper right first molar. They explain the diagnosis in plain language: the nerve inside the tooth has become severely inflamed and cannot heal on its own, which is why the pain has been persistent and worsening. They clarify that a filling alone will not resolve this because the problem lies deep inside the tooth's nerve, not just the outer surface.

The candidate then describes the root canal procedure in accessible terms: the tooth is numbed with local anaesthetic, the inflamed nerve tissue is removed, the canals are carefully cleaned, shaped, and disinfected, and then sealed with a filling material. A crown or permanent restoration is placed afterwards to protect the tooth. They note that treatment typically requires two to three appointments.

Benefits are clearly outlined: relief of pain, preservation of the natural tooth, and avoidance of extraction. The candidate then addresses material risks specific to this case, including the proximity of the roots to the maxillary sinus — explaining that there is a small risk of communication with the sinus during treatment, which would be managed appropriately. Other risks discussed include treatment failure (approximately 5–10%), persistent or new pain, instrument separation within the canal, root perforation, tooth fracture, and the possible need for retreatment or a minor surgical procedure called an apicectomy.

Alternatives are presented: extraction with or without replacement via implant, bridge, or denture, or no treatment with the risk of spreading infection and worsening pain. Pain management is addressed directly, reassuring the patient that modern root canal treatment is performed under effective local anaesthesia and is generally comfortable, with over-the-counter analgesia such as paracetamol or ibuprofen recommended post-operatively. The candidate checks understanding, invites questions, and confirms the patient's willingness to proceed.

PQ-10 Peg Lateral Incisor — Explain and Plan

Restorative dentistry · 11 minutes · Cluster 2

The station. *Miss Nguyen is a new patient at your clinic, attending today complaining about the appearance of her front tooth. She works in the pub and her partner thinks she has vampire tooth. MH; patient has anxiety in general and taking diazepam 10mg twice/day. Diagnose and manage patient concern.*

An excellent candidate would begin by warmly greeting Miss Nguyen, establishing rapport, and inviting her to describe her concern in her own words. They would acknowledge her self-consciousness and validate that this is a common and very treatable condition. After confirming her medical history — noting her diazepam use for anxiety and considering its implications for treatment planning and sedation — the candidate would explain in plain language that her 'vampire tooth' appearance is caused by a peg lateral incisor, a developmental variation where the tooth simply grew smaller than usual, and that this is not a sign of disease or poor oral hygiene. The candidate would clarify that both upper lateral incisors appear to be affected, which is actually common when this occurs. They would then outline three main treatment pathways: first, composite resin build-up — a conservative, tooth-friendly option where tooth-coloured filling material is shaped around the existing tooth to create a normal appearance, requiring little to no drilling, fully reversible, and costing approximately $200–$400 per tooth; second, porcelain veneers — thin ceramic shells bonded to the front surface, requiring minimal but irreversible enamel reduction, offering superior aesthetics and longevity of 10–15 years, at approximately $1,500–$2,500 per tooth; and third, full crowns — generally not recommended here as they require significant tooth reduction and are disproportionate to the clinical need. The candidate would address her concern about damaging her teeth directly, reassuring her that composite build-up preserves virtually all natural tooth structure.

They would note that her private health insurance may contribute to composite restorations. Given her anxiety, the candidate would adopt a calm, unhurried approach, offer written information, and involve her in shared decision-making by recommending composite build-up as a reversible starting point, with the option to upgrade to veneers later. A follow-up treatment planning appointment would be offered.

broucher explaining treatment options also seeking help from someone close to decide the treatment option and treatment descion should come from her as shes seeking treatment after a comment from her boyfriend who shes dating since 3 months(domestic violence of dominating nature can be suspected here but not sppoken directly about)

PQ-30 Amalgam Filling Complication: Post-Operative Pain Assessment

Restorative dentistry · 11 minutes · Cluster 3

The station. *Scarlett Jones is a regular patient in your clinic. She is attending today complaining of severe pain related to one of her lower left first molar tooth. She had an amalgam filling done by one of your colleagues 2 weeks ago while you were away. She is very upset, and she hasn't been able to sleep for the past 2 nights. You took an Xray and found out that there are some caries under amalgam filling and apical radiolucency associated with that tooth and because of which she is experiencing pain. How will you manage her concerns and give her appropriate treatment options?*

An excellent candidate would begin by warmly acknowledging Scarlett's distress and validating her experience before taking a structured pain history. They would ask about the onset, character (sharp, throbbing, spontaneous), duration of pain episodes, and specific triggers including cold, heat, and chewing. They would confirm the history of two nights without sleep and current analgesia use. Before proceeding, they would confirm medical history, current medications, and allergies.

On examination, the candidate would perform percussion testing on tooth 36 and adjacent control teeth, palpation of the surrounding tissues, and thermal sensitivity testing using cold (ethyl chloride or ice) and heat, noting the lingering response to cold exceeding 10 seconds and tenderness to percussion. They would review the radiograph, noting the restoration is within 0.5 mm of the pulp and confirming the absence of periapical radiolucency at this stage.

The candidate would explain the diagnosis of irreversible pulpitis in plain language — that the nerve inside the tooth has become inflamed beyond the point where it can recover, most likely because the decay was very deep and close to the nerve before the filling was placed. They would clarify this is a recognised complication of deep restorations, not necessarily a result of negligence.

They would present two definitive treatment options: root canal treatment to save the tooth, explaining the process, likely two to three appointments, and associated costs; or extraction with discussion of replacement options such as an implant or bridge. They would recommend root canal treatment as the preferred option to preserve the tooth.

For interim management, they would prescribe or recommend appropriate analgesia such as ibuprofen 400 mg with paracetamol 500–1000 mg alternating, and arrange a timely appointment for root canal treatment or referral to an endodontist.

Regarding the colleague, the candidate would respond professionally and empathetically, acknowledging Scarlett's frustration without apportioning blame, explaining that deep decay can sometimes cause the nerve to become inflamed even after a technically sound filling. They would document findings thoroughly and advise the colleague as a matter of professional courtesy and clinical governance.

PQ-35 Amalgam Replacement Discussion

Restorative dentistry · 11 minutes · Cluster 2

The station. *Mr Walcott a 55-year-old medically fit patient, attending your clinic today concerned about mercury poisoning due to his amalgam filling that has been present in his mouth for the past 10 years by his previous dentist. He wants to replace all his amalgam fillings with composite. There is no fracture of restoration, pain, or sensitivity. Explain how would you manage this patient and how you plan to address his concerns?*

I would begin by warmly welcoming Mr Walcott and acknowledging his concerns without dismissal, recognising that anxiety about amalgam safety is understandable given the volume of information circulating on social media. I would ask him to share what he has read or been told, and explore whether he is experiencing any symptoms he attributes to his fillings. After listening carefully, I would explain that the current Australian evidence-based position — supported by the NHMRC and the Australian Dental Association — is that dental amalgam is safe and effective for the vast majority of patients. The mercury in amalgam is in a stable, bound form (mercury sulphide) and is not equivalent to the organic methylmercury associated with toxicity. I would clarify that his fillings are ten years old, well-established, and that the greatest release of mercury vapour actually occurs during placement and removal — not during routine function. I would then explain the risks of unnecessary replacement: irreversible loss of healthy tooth structure, risk of pulpal irritation or damage, the possibility of new restoration failure, and financial cost, noting that composite restorations in posterior load-bearing areas have a shorter average lifespan than well-maintained amalgam. I would identify that valid clinical indications for replacement include fracture, secondary caries, poor marginal integrity, documented mercury allergy, or aesthetic concerns in visible areas — none of which appear to apply here based on his history. I would discuss composite resin as a viable alternative material, acknowledging its aesthetic advantages while being transparent about its limitations in high-stress posterior sites. Finally, I would reach a shared decision with Mr Walcott: recommending monitoring and regular review rather than immediate replacement, providing him with reliable resources such as the ADA patient information and NHMRC guidelines, and inviting him to return with any further questions. I would document the discussion and his informed decision.

PQ-38 International Student with large silver filling

Restorative dentistry · 11 minutes · Cluster 1

The station. *Mr Gamage, 45-year-old overseas student, came to Australia 2–3 years ago. Chief Complaint: pain from one of his teeth on the lower right side for the past 2 weeks. He cannot localise a particular tooth, but biting on that side makes it worse. Examination Findings: large MOD amalgam restoration on 46, placed around 20 years ago. Clinical Image: intraoral photo showing a large, aged MOD amalgam restoration on tooth 46 with visible fracture lines running through the amalgam. Task: gather information, explain the investigations you would carry out, give a possible diagnosis, and address the patient's concerns.*

A strong candidate recognises early that this is not simply a failing filling. They take a focused pain history — two weeks, lower right, cannot be localised, worse on biting — and then go looking for the mechanism rather than stopping there. Asking about diet, or about exactly when the pain began, elicits that Mr Gamage was biting on ice when the tooth cracked.

They then ask about waking symptoms, which is where the station really opens up: sore muscles and headaches on waking, sometimes. A candidate who never asks this misses the entire TMJ and bruxism thread that the case is built around.

Getting to the cause takes persistence. Mr Gamage is cranky, tired and working night shifts as a security guard with pub work on top of study, and he will not hand over the stress on a single closed question. The candidate asks openly, gives him room, and comes back to it — reaching the burnout picture underneath rather than accepting the first deflection.

On investigations they are specific rather than vague: muscle palpation, a bite test, an IOPA or bitewing, and transillumination to look for the crack. When Mr Gamage asks directly how anyone would know whether the tooth has actually cracked, they answer him plainly instead of deferring. They work towards a clear view of whether the pulpitis is reversible or irreversible rather than leaving pulpal status undefined.

Management is staged. In the short term: a custom-made full-coverage splint, and removal of the existing filling with temporisation. In the longer term: a crown, or root canal treatment followed by a crown, depending on the pulpal diagnosis reached.

Finally, they treat the working pattern as part of the problem, not background colour. Health promotion around stress management and preventing burnout belongs in the answer, because the tooth is the presenting complaint and the way Mr Gamage is living is the reason it presented. Throughout, they stay patient with a man who is short-tempered because he is exhausted and sore, rather than matching his tone.

A candidate performing at the top of the band also handles the questions Mr Gamage puts back to them — why the pain comes on releasing rather than biting down (the release causes a snap-back across the crack), how a crack would actually be identified, whether it would show on an X-ray, and how he would even know whether he grinds. They answer these directly rather than deferring, and they use the fact that he cannot localise the pain as positive diagnostic information: poorly localised pain points away from apical periodontitis and towards a cracked tooth, since proprioception would let him pinpoint the tooth if the periapex were involved.

PQ-40 Assessment and Management of Dental Fluorosis

Restorative dentistry · 11 minutes · Cluster 2

> ***The station.*** *Isla, 15-year-old girl. Chief Complaint: Complaining about white and brown patches on all of her teeth. Examination Findings: White and brown spots throughout the mouth. History: Lived in a rural area until she was 10, then moved to boarding school. Took fluoride tablets until age of 10. Doesn't want any drilling or local anaesthesia. Wants teeth fixed ASAP — has a photoshoot at sister's wedding in a few weeks. Clinical Image: Frontal photo of upper and lower teeth showing generalised white and brown/yellow mottled staining across all visible teeth — classic dental fluorosis presentation. Task: Explain diagnosis, treatment planning and management.*

An excellent candidate would begin by warmly greeting Isla and her parent, then take a focused history exploring fluoride exposure during the critical period of tooth development — birth to age eight. They would ask about the water source in the rural area where Isla lived until age ten, whether fluoride tablets were taken alongside fluoridated water, and whether she swallowed toothpaste as a young child. They would also ask about any childhood illnesses, fevers, medications such as tetracyclines, or dental trauma to differentiate fluorosis from other enamel defects such as molar-incisor hypomineralisation or amelogenesis imperfecta.

The candidate would then explain the diagnosis clearly and sensitively: the white and brown patches are dental fluorosis, caused by excess fluoride during enamel formation in early childhood. They would reassure Isla and her parent that the enamel is structurally sound, the teeth are not weaker, and there is no increased risk of decay. The candidate would acknowledge that multiple fluoride sources — rural water, fluoride tablets, and toothpaste — likely contributed, and would avoid assigning blame, particularly reassuring the parent that this was not a result of negligence.

Addressing Isla's immediate concern about the wedding photoshoot, the candidate would explain that several options exist depending on severity. For mild-to-moderate fluorosis with intact enamel, microabrasion combined with home bleaching can reduce the appearance of white spots. Resin infiltration (Icon) is a minimally invasive option that masks white spot lesions without drilling or anaesthesia — directly addressing Isla's preference. Composite veneers or porcelain veneers would be deferred until adulthood given her age. The candidate would involve Isla in the decision, explain realistic timeframes, and note that resin infiltration could potentially be completed before the wedding. They would also recommend monitoring and reassess at eighteen for more definitive options if desired.

PQ-52 Vague Pain on the Left Side — Interproximal Caries & Open Contact (26/27)

Restorative dentistry · 11 minutes · Cluster 2

The station. *Female, new patient attending for routine check-up and clean. Chief Complaint: Food lodgement between teeth 26 and 27. Vague pain/aching sensation on the upper left side, mentioned at last appointment with previous dentist. Examination Findings: Radiograph provided showing: radiolucency involving the contact point between 26 and 27 (interproximal caries extending into dentine), affecting the marginal bone; Gap/open contact between 26 and 27; Temperature testing: 35°C response noted; Generalised amalgam restorations present. Task: Carry out history taking and examination. Determine investigations. Establish a working diagnosis and explain findings to the patient.*

An excellent candidate begins by warmly welcoming Maria as a new patient and establishing rapport before taking a focused history. They ask open-ended questions about the vague upper left aching — eliciting onset (approximately eight months ago, though closer to eighteen months on gentle probing), character (dull, aching rather than sharp or spontaneous), triggers (cold, sweet foods), and critically, whether the pain lingers after the stimulus is removed. They specifically ask about food packing between the upper left back teeth, confirming the chief complaint. The candidate then asks directly about parafunctional habits — grinding, clenching, or morning jaw soreness — eliciting Maria's disclosure that her partner has noticed nocturnal grinding and that she wakes with a sore jaw. This is integrated into the management plan. On examining the bitewing radiograph, the candidate reads it systematically: confirming image quality, correct angulation, and patient identification before describing findings. They identify a well-defined radiolucency at the distal surface of 26 extending through enamel into dentine, an open contact between 26 and 27, and subtle loss of crestal bone height at the interproximal crest. They note the existing amalgam restorations appear radiographically intact, explaining why the lesion was not visible clinically. The candidate correlates the prolonged thermal response at 35°C with reversible pulpitis, clearly distinguishing this from irreversible pulpitis, while acknowledging that monitoring is essential and root canal treatment for 26 cannot yet be excluded. When Maria asks why her previous dentist missed this, the candidate validates her concern empathetically — explaining that interproximal caries hidden beneath a contact point is invisible to clinical examination alone and that bitewing radiographs are specifically required to detect such lesions — without criticising the previous practitioner. Management proposed includes restoration of the distal surface of 26 with re-establishment of the contact point, periodontal assessment of the 26/27 region, pulp vitality monitoring with a review appointment, an occlusal splint given the bruxism history, and a six-month recall bitewing radiograph.

PQ-57 16-Year-Old with Rampant Caries & Erosion — Dietary + Lifestyle Causes

Restorative dentistry · 11 minutes · Cluster 3

The station. *Sam, 16-year-old boy, attending with his father. Chief Complaint: Sensitivity to cold on several teeth. Father is accompanying him. History: Regular patient — seen 1 year ago, teeth were ok at that time; Started a new part-time job as an apprentice approximately 1 year ago; High sugar intake; drinks a lot of soft drink during work; Dietary change coincides with onset of sensitivity. Examination Findings / Clinical Images: Cervical caries on tooth 12 and white spot lesions (early caries) on anterior teeth; White chalky patches (enamel demineralisation) on multiple teeth; Caries at cervical margins of multiple posterior teeth, also affecting the marginal bone in some areas; Erosion pattern…*

An excellent candidate would begin by warmly greeting both Sam and his father, then immediately establish Sam as the primary patient by directing the first question to him: 'Sam, can you tell me in your own words what's been bothering you?' The candidate would acknowledge Mr Tran's presence and concern early, giving him a defined supportive role while consistently redirecting clinical questions to Sam. History-taking would systematically quantify acidic drink consumption — asking specifically about energy drinks and soft drinks at work, sports drinks after exercise, and what Sam drinks at home versus on-site. When Sam initially underreports, the candidate would probe gently: 'What do you and your workmates usually grab during breaks?' to elicit the true two-to-four cans per day figure. Crucially, the candidate would ask directly: 'Do you ever brush your teeth at work, or after having a drink?' — uncovering Sam's habit of brushing immediately after acid exposure and correcting it clearly: 'Brushing straight after an acidic drink actually scrubs away softened enamel — we need to wait at least 30 to 60 minutes first.' The candidate would rule out GORD and eating disorders as alternative acid sources. Clinical findings would be explained by distinguishing caries — a bacterial process where mouth bacteria ferment sugar and produce acid — from erosion, a direct chemical dissolving of enamel by dietary acid, independent of bacteria. Both are present and both are being driven by the same drinks. The twelve-month trajectory would be used motivationally: 'One year ago your teeth were fine. In twelve months you've developed multiple early lesions and one cavity — if this continues, the damage in the next year will be even greater.' The management plan would include same-day fluoride varnish, prescription of Colgate Neutrafluor 5000 ppm toothpaste, advice to replace between-meal drinks with fluoridated tap water, fluoride mouthrinse at a separate time, and restorations for teeth 12 and 26. White spot lesions would be monitored conservatively. A three-month recall would be scheduled.

Prosthodontics

5 stations

PQ-3 Management of Denture Stomatitis in a Diabetic Patient

Prosthodontics · 11 minutes · Cluster 2

The station. *Mrs Smart 65 years old, wearing upper denture for 20 years. Current denture is 20 years old. She complains of broken denture. Denture falls during yawning and eating. She has few teeth on the lower jaw but doesn't wear any denture on lower. On examination you found red inflamed palate, upper denture has buccal flange overextension, all the teeth worn out. Consult with the pt about the management.*

An excellent candidate would begin by warmly introducing themselves and establishing rapport before taking a focused history. They would ask about the duration and fit of the current denture, overnight wear habits, and the patient's denture cleaning routine — specifically whether she soaks her dentures or uses denture cleaner. They would then explore her medical history, asking directly about her diabetes, how it is managed, and her most recent HbA1c result. They would also ask about dry mouth symptoms and any medications that might contribute to xerostomia, as well as dietary habits such as sugar intake.

Based on the history and clinical findings — diffuse palatal erythema beneath a poorly fitting, plaque-laden upper denture in a patient with poorly controlled Type 2 diabetes who wears her denture overnight — the candidate would diagnose Denture stomatitis, most likely candidal in origin. They would clearly explain to Mrs Smart that the redness and soreness are caused by a fungal infection called Candida, and that several factors are working together to cause this: her denture acting as a reservoir for the fungus, wearing it overnight preventing the palate from recovering, infrequent cleaning allowing plaque and organisms to accumulate, reduced saliva flow reducing natural oral defences, and poorly controlled blood sugar providing an environment in which Candida thrives.

Management for I'll fitting denture offered as chairside or lab relining explaining the advantages and disadvantages of both

Management would be explained in clear stages. First, she must stop wearing her denture when partner is not around wash it with soap and water Denture stomatitis is something you have noticed it's not a presenting complain don't jump to denture stomatitis without addressing their concerns first also explain why they should delay the denture as she has denture stomatitis and can affect the fit of denture. She should brush the denture with a soft brush and rinse thoroughly. Use a towel underneath to prevent breakage in future

If they don't like soap offer denture cleaing tablets

Second, the candidate would would reviev after 2 days and if the diabetes is unstable then this requires specialist advised (candida infection in immunocompromised patient requires aggressive treatment and they may prescribe systemic anti fungal) also if diabetes is normal review after 1 month if there's no improvement Prescribe Anti fungal gel,

Angular Chelitis is a exxtension of what is happening inside our month. Management includes numbing gel swab test to confirm if it due to candida (anti fungal cream) blood test to confirm if it is due to deficiency.

Third, they would address the overextended buccal flange and poor retention, explaining that a new denture will be needed once the infection resolves and her diabetes is better controlled. Finally, they would recommend she see her GP to review her HbA1c, explaining that improving blood sugar control is essential for the infection to resolve and for any future dental treatment to succeed.

PQ-7 Replacement Options After a Posterior Extraction

Prosthodontics · 11 minutes · Cluster 3

The station. *70 years old lady, a new patient in your practice, is a regular dental attendee. You have done some scaling and taken an x-ray; you inform the patient they don't have any new cavities and her gums are healthy. Then she casually asks what replacement options are there for the gap between 34 and 36 (missing 35) present between her teeth. Intraoral picture and x-ray of the left side given showing the gap is small — implant suitability will need to be confirmed with further imaging (e.g. CBCT). On x-ray, 36 has an under-filled RCT and a metallic cap. Management and replacement options.*

An excellent candidate would begin by warmly acknowledging the patient's question and taking a brief focused history before launching into options. They would confirm that tooth 35 was extracted four weeks ago due to a vertical root fracture, that healing has been uneventful, and that the patient has no pain or swelling. They would note the adjacent teeth 35 and 37 are minimally restored, the patient's well-controlled hypertension on Perindopril, and her concerns about cost, appearance, and function. The candidate would then explain that doing nothing is an option but carries real consequences — the opposing tooth (45) may over-erupt, and teeth 34 and 36 may drift or tilt into the space over time, compromising chewing and making future replacement harder. They would present three main replacement options. First, a dental implant: a titanium fixture placed into the jaw bone after adequate healing (typically three to six months post-extraction), with a crown placed on top. It does not involve preparing adjacent teeth, has the best long-term prognosis (15-plus years with good care), but involves surgery, a longer treatment timeline, and the highest cost (approximately $4,000–$6,000 out of pocket in Australia). Second, a three-unit fixed bridge using teeth 34 and 36 as abutments: a fixed, non-removable option that restores function and aesthetics well, is completed in two to three appointments, and is less costly than an implant, but requires irreversible preparation of two otherwise sound teeth and demands careful flossing technique underneath. Third, a removable partial denture: the most affordable and least invasive option, but removable, potentially bulky, and often the least preferred by patients long-term. The candidate would acknowledge the patient's reluctance toward a denture, validate her preference for something fixed, and recommend a CBCT or OPG assessment to confirm bone volume for implant planning. They would invite questions and summarise next steps clearly.

PQ-27 Assessment and Counselling for Dental Implant Treatment

Prosthodontics · 11 minutes · Cluster 1

> ***The station.*** *Patient fit and healthy wants implants in all back teeth, take history and investigate to see if she is suitable for implants. Prosthetic heart valve replacement surgery 2 years ago, takes some medication for blood doesn't know what it is — 2 little pills. Gather information and explain necessary investigations.*

An excellent candidate would begin by warmly introducing themselves and confirming the patient's identity and chief concern. They would take a structured medical history, starting with open questions before moving to targeted enquiry. On identifying the prosthetic heart valve, they would ask about the specific anticoagulant medication — recognising that 'two little pills' likely represents warfarin — and explain that an INR check will be needed before any surgical procedure. They would then systematically ask about all current medications, specifically prompting about tablets for bone health or osteoporosis, thereby eliciting the alendronate use. They would note the three-year duration and explain that bisphosphonate therapy carries a risk of medication-related osteonecrosis of the jaw (MRONJ) following implant surgery, and that GP liaison is essential before proceeding. On identifying type 2 diabetes managed with metformin, they would ask about current HbA1c, frequency of monitoring, and when the patient last saw their GP, noting that an HbA1c above 7.5–8% is associated with impaired healing and reduced implant success rates. They would explain that optimising glycaemic control prior to surgery is a prerequisite. Regarding investigations, they would request an OPG as a minimum, with CBCT if bone volume assessment is required, and explain that immediate implant placement is not appropriate — a full assessment of bone quantity and quality is needed first. They would outline the implant process in lay terms: assessment, possible bone grafting, implant placement, a healing period of three to six months for osseointegration, then crown placement. They would present alternatives including a fixed bridge or removable partial denture, invite the patient's preferences, and check understanding throughout using teach-back. They would close by summarising next steps and arranging GP correspondence.

NEW Flabby Ridge — New Denture for a 20-Year-Old Loose RPD

Prosthodontics · 11 minutes · Cluster 3

> ***The station.*** *Mr Warren James is here today looking for a new denture, as his current RPD is 20 years old and loose. On examination, in the upper jaw only two teeth are present — 17 and 27, both grade 2 mobile — and the upper ridge is flabby. The lower arch has all natural teeth (photograph given, front view). He is medically fit and healthy. Address and manage the patient's concerns.*

An excellent candidate starts by taking Mr James's request seriously — he has come in for a new denture and should not be told bluntly that he cannot have one. They establish how the current denture is failing and, importantly, why he is attending now: the teeth are no longer visible in the worn old denture, so this is as much an aesthetic complaint as a functional one.

They take a history that uncovers the accident as the reason for the tooth loss, and they do not accept "medically fit and healthy" as the end of the medical history — they explore the blood test six months ago and the multivitamins he is taking.

On examination they identify the central problem and, crucially, explain its cause: the upper ridge has become flabby because of excessive forces from the full complement of natural lower teeth acting on the upper anterior area. They state the diagnosis of flabby ridge explicitly, and give a prognosis for the two remaining upper teeth, 17 and 27, both grade 2 mobile. They notice, or ask about, the angular cheilitis and the skin fold at the corner of the mouth, and connect this to a loss of vertical dimension rather than treating it as an incidental finding.

The core of the answer is an honest explanation delivered without alarm: they are happy to make a new denture, but it will not be straightforward. If a new denture is simply built like the old one with clasps on 17 and 27, the flabby anterior ridge means tipping forces at the back will, in time, cost him those two remaining teeth. The better option is to remove them and provide a complete denture, which has a better prospect of success. They can discuss the impression technique a flabby ridge requires, rather than treating it as a routine impression.

They set out the longer view: an implant-supported overdenture or bridgework is the recommendation, because the forces at play will continue to resorb bone, and the longer this is left the more difficult those options become. A transitional denture can be provided in the meantime.

On referral they are specific. This goes to a prosthodontist — the specialty that handles complex removable and implant prosthodontics, which is exactly what a flabby ridge, a selective-pressure impression and an implant-supported overdenture require. If implants proceed, placement may involve an oral and maxillofacial surgeon or a periodontist, with the prosthodontist restoring. Crucially, they frame the referral as the right first call rather than something to try after a conventional denture has failed: the situation is already compromised, and attempting a straightforward remake risks the remaining teeth and the bone that the implant options depend on.

If Mr James pushes back on cost — as he reasonably might — they do not simply accept it and move on. They restate plainly what the clinical risk of that choice is, offer what can be done within his means, and make sure the decision is genuinely informed rather than defaulted into.

Finally — and this is what separates a strong candidate — they answer the question of whether they can do this themselves as a general dentist candidly, naming the specific challenges they will face rather than presenting the plan as simple. Mr James leaves understanding why the straightforward answer he wanted is not the one that serves him best.

NEW Replacement Options — Diabetic Patient with a Wedding in Two Weeks

Prosthodontics · 11 minutes · Cluster 3

> ***The station.*** *Naseem, 53, presents wanting to replace missing teeth. Missing: 26, 36, 46. Tooth 16 is slightly supra-erupted. Medical history includes diabetes, and vitamin D has been prescribed by the GP. There is a family wedding in two weeks. Take a history give short term replacement options*

A strong candidate greets Naseem warmly and opens by finding out what is driving the visit now — the wedding in two weeks — while gently establishing why the teeth have not been replaced before this point. They take a structured history rather than jumping to options: how often Naseem attends and why, why 26, 36 and 46 were removed, and what the current oral hygiene routine looks like.

On the medical history they do not simply record "diabetic". They ask how the diabetes is managed, whether recent blood tests have been done, and about the vitamin D the GP has prescribed, connecting irregular attendance to the wider pattern of care where relevant. The social history covers smoking, alcohol and — importantly — finances, since finances will decide what is genuinely achievable.

Before recommending anything, the candidate asks Naseem what he has in mind: removable or fixed. They then set out the options honestly. They identify the supra-erupted 16 and explain why it matters — a resin-bonded bridge opposing a supra-erupted tooth can be dislodged — and discuss whether to reduce the tooth prosthodontically or intrude it orthodontically, noting that diabetes already places him at higher risk and weighs against prolonged intervention.

Crucially, they explain why diabetes changes the long-term picture: reduced immunity and dry mouth make a poor environment for any prosthesis, a prosthesis is a foreign body that carries real risk of recurrent caries, and meticulous hygiene, home fluoride and more frequent reviews are what keep that risk manageable.

They use the wedding to motivate rather than to rush, saying clearly that definitive work will not be finished in two weeks while agreeing what can realistically be started. A plan along the lines of a resin-bonded bridge at 26 with a removable partial denture for 36 and 46 is reached jointly, with review arranged.

Paediatric dentistry

9 stations

PQ-2 Caries Risk Assessment: 8-Year-Old

Paediatric dentistry · 11 minutes · Cluster 1

The station. *Mehmat Khan is a 8 years old cooperative child came in your practice with mother. Mother is worried about his teeth turning black. single mom busy at work, Grandparents mostly take care oh Mehmat. On examination you found a lot of carious teeth. caries on both permanent molars Consult with the mother about her concerns. Intraoral picture given for lower jaw, caries on both permanent molars, E/D/C's. Consult with the mother about caries risk assessment and treatment.*

This station is not really about caries. It is about finding out who actually feeds and supervises this child, and about being told "no fluoride" and still producing a plan.

OPEN BY TAKING THE GUILT OFF THE TABLE

The parent opens with "I feel terrible, I don't know what I did wrong." Answer that before anything else: "You've brought him in, and that's the thing that matters. This is really common and it's almost never one thing somebody did wrong — let's work out what's driving it and fix it together." A candidate who launches into diet questioning without acknowledging that line has already lost the room.

THE FINDING THE PARENT HAS MISSED

The parent is worried about whether this will "affect his permanent teeth." The permanent first molars are already erupted and already showing pit-and-fissure decay. That is the single most important thing to surface, gently: the damage is not a future risk, it has started. Say it without alarm, but say it clearly — the whole treatment plan depends on the parent understanding it.

FIND OUT WHO IS ACTUALLY LOOKING AFTER HIM

The parent works and the grandparents do most of the day-to-day care. So a diet history taken from the parent alone is incomplete, and they will tell you as much if you ask. You need:

- What is he eating and drinking at his grandparents', at school, at daycare — not just at home.
- FREQUENCY, not quantity. Frequent snacking is the driver here, and it is the number that matters.
- Fluid intake — he does not drink much water. Less water means less salivary clearance, so sugar sits on the teeth longer and the mouth stays acidic. That is worth explaining in plain terms, because it makes "give him water" feel like treatment rather than nagging.
- Anything given at bedtime.

The practical point: everyone who feeds him has to be on the same plan. A diet sheet the parent follows and the grandparents do not is not a plan.

THE BRUSHING HISTORY — ASK HOW, NOT WHETHER

He brushes with the door shut and will not let anyone supervise. An 8-year-old does not have the manual dexterity or the attention to brush adequately unsupervised, and "does he brush?" gets you a yes that means nothing. Ask who watches, how long, what toothpaste, what time, and what happens after. Then work with his need for independence rather than against it: he brushes himself, and a parent or grandparent checks afterwards — disclosing tablets, a quick look, a chart. Frame it as him being in charge and an adult signing off, not as taking the job away from him.

THE HARD PART — "NO FLUORIDE"

The family declines fluoride on religious grounds. Do not argue, do not repeat the offer three times, and do not treat it as ignorance to be corrected. Explore what specifically the concern is, offer to provide information, and respect the decision. Then adapt:

- Note that glass ionomer is also off the table if the objection is to fluoride at all, since GIC releases fluoride. Say so honestly rather than slipping it in.
- What is still available: diet modification, plaque control, resin fissure sealants on the permanent molars, CPP-ACP (casein-based — check it is acceptable for the family), xylitol, water intake, and restorative materials without fluoride release such as composite, and stainless steel crowns or Hall crowns for the primary molars.
- Because you have lost your single most effective preventive agent, everything else has to work harder and the recall interval has to be shorter. Say that out loud — it explains why you are asking for more visits, not fewer.

THE QUESTION THEY WILL ASK — "CAN'T WE JUST PULL THEM OUT, THEY'RE BABY TEETH?"

No, and explain why rather than just refusing. Primary molars hold space; taking them out early lets the permanent teeth drift and crowd, and can create an orthodontic problem that costs far more than restoring them now. Also — and this is the point that lands — extracting baby teeth does nothing about the permanent molars, which are the teeth already decaying.

TG'S FOUR-STEP CARIES MANAGEMENT — USE IT AS YOUR STRUCTURE

- Diet modification — reduce frequency of sugar exposure, everyone involved.
- Plaque reduction — supervised or checked brushing, technique, timing.
- Tooth surface modification — sealants, and normally fluoride; here, the non-fluoride alternatives above.
- Saliva modification — water intake, and address anything reducing flow.

Naming the four and then populating each one is a clean, examinable way to show a complete preventive plan rather than a list of tips.

INVESTIGATIONS AND TREATMENT

Full charting, bitewings, assessment for pulpal involvement and any pain or sepsis. Then a staged plan: stabilise the active lesions, restore, seal the permanent molars, and set a short recall (around three months given the risk profile and the declined fluoride).

THE BARRIERS — ASK, DON'T ASSUME

They have not been before because of cost and because a single working parent cannot easily get here. Both are solvable and neither is a character flaw. Raise the Child Dental Benefits Schedule — an eligible child gets a capped amount over two consecutive calendar years, which covers a lot of what this child needs. Offer appointment times that fit around work, and stage treatment so it is affordable. A plan the family cannot attend is not a plan.

COMMON WAYS TO FAIL THIS STATION

Never asking who else feeds him. Taking "he brushes" at face value. Missing that the permanent molars are already involved. Arguing with the fluoride refusal, or ignoring it and prescribing fluoride varnish anyway. Offering GIC without noticing it releases fluoride. Agreeing to extract the primary teeth as a shortcut. Producing a preventive plan with no mention of cost or of the grandparents.

PQ-8 Congenital Syphilis Presenting in a Child

Paediatric dentistry · 11 minutes · Cluster 1

The station. *Stevie Umanarah is an 8-year-old patient at your clinic, visiting today with her foster carer Mrs Kaur. The carer is mainly concerned about the appearance of her teeth. You examined her and took an intraoral picture showing notched, upper incisors. has difficulty holding things has partial eye sight and hearing loss. Explain the necessary investigations and address the carer's concerns.*

This station looks like a paediatric aesthetics case and is actually about consent and confidentiality. The dental findings are the easy part; the marks are in knowing what you are allowed to say and to whom.

WHAT YOU ARE LOOKING AT

Notched, screwdriver-shaped permanent incisors (Hutchinson's incisors) and mulberry molars. Put those alongside the partial eyesight and the hearing loss given in the brief and you have the classic triad of late congenital syphilis — Hutchinson's teeth, interstitial keratitis and sensorineural hearing loss. You should recognise it. That is not the same as announcing it.

THE CONFIDENTIALITY TRAP — THIS IS THE STATION

Mrs Kaur is a SHORT-TERM foster carer, and she has not been told Stevie's underlying diagnosis. She knows only that Stevie has "some developmental issues."

It is not your place to hand her that diagnosis across the surgery. You did not make it, you have not seen the medical records, and disclosing it to someone the treating team has chosen not to inform is a breach of the child's confidentiality. Describe what you can see without naming the condition:

"What I can see is that these teeth didn't form in the usual way while they were developing — the shape and the surface are different from normal. That happens with a number of conditions that affect a child early on. I don't have Stevie's medical records in front of me, so rather than guess I'd like to contact the organisation and confirm what's already known and what's been shared with you."

That sentence — "let's confirm with the organisation" — is the one the station is looking for.

THE CONSENT TRAP — AND THE MONEY

A short-term carer has authority to consent to EXAMINATION and INVESTIGATION. She does NOT have authority to consent to treatment. Treatment consent has to come from the organisation or statutory guardian, and a referral to hospital or to a specialist needs its own separate approval.

Mrs Kaur will push. There is a wedding coming up, she wants Stevie's teeth sorted, and she will offer to pay out of her own pocket. Handle it kindly and hold the line: paying for something does not create the legal authority to consent to it. Willingness to fund treatment and authority to agree to it are two different things, and only one of them is yours to accept.

"I can hear how much you want this sorted for her, and I do want to help. The difficulty isn't the money — it's that consent for treatment has to come from the organisation that has guardianship, not from us. What I can do today is examine her, take the records we need, and get that approval moving so we're ready to go as soon as it comes through."

WHAT YOU CAN DO TODAY

Examination and investigations are within her consent, so use the appointment properly:

- Extra-oral and intra-oral examination.
- Radiographs — bitewings and an OPG.
- Age-appropriate periodontal screening.
- Clinical photographs.

- Diet chart.

That gives the organisation a complete picture to consent against, and means nothing is wasted.

THE DEXTERITY QUESTION EVERYONE FORGETS

Stevie has difficulty holding things, plus partial sight. So ask the obvious follow-up: how is she actually brushing? Then adapt — a modified or built-up handle, a powered brush, positioning, and carer assistance for the parts she cannot manage. Her caries risk is already high; a toothbrush she cannot grip is a live problem, not a footnote.

EMPOWER THE CHILD, NOT JUST THE CARER

Stevie will move between carers. Every handover is a chance for her routine to be lost. So teach HER — demonstrate at her level, let her practise, give her something she owns and can carry to the next placement. Speak to her directly and at an age-appropriate level rather than talking over her to the adult for ten minutes. This is the part candidates skip, and it is explicitly in the marking.

THE OTHER CONCERNS

She will raise the spacing and the puffy gums if asked. The spacing and any jaw discrepancy are complex, need specialist input, need a separate referral and separate consent, and are likely a hospital pathway. Worth knowing: orthodontic treatment is not normally covered under the Child Dental Benefits Schedule, but funding pathways do exist for children with developmental conditions — so do not tell the carer it is simply unfunded.

COMMON WAYS TO FAIL THIS STATION

Naming congenital syphilis to the carer. Accepting her consent for treatment. Accepting her offer to pay as though that settles it. Never asking what kind of carer she is or what she has been told. Missing the dexterity issue. Talking only to the adult and never to Stevie. Promising treatment dates you have no authority to promise.

PQ-12 12-Year-Old with trauma

Paediatric dentistry · 11 minutes · Cluster 3

> ***The station.*** *12 year old Girl has injuries during hockey. Didn't wear mouth guard, 21 fracture to enamel dentin area no exposed the pulp. 11 displaced palatally showed no response to vitality test. No X-ray taken. provide short- and long-term solution.manage patients concerns.*

An excellent candidate would begin by introducing themselves warmly to both Sarah and her mother, acknowledging Sarah's distress and reassuring her before proceeding. They would take a focused trauma history covering the time and mechanism of injury, whether a mouthguard was worn, presence of pain (rated 6/10, worse with cold air), any loss of consciousness, headache, nausea, or vomiting, and confirm tetanus immunisation status is current. They would note the fracture fragment was not retrieved and that bleeding from the lip has stopped.

On examination, they would perform a systematic extra-oral assessment of the face, TMJ, and lip laceration, then an intra-oral assessment including soft tissues, occlusion, mobility testing, percussion, and sensibility testing. For tooth 11, they would identify a complicated crown fracture with approximately 2mm pulp exposure, tenderness to percussion, and a vital response. For tooth 21, they would identify an uncomplicated crown fracture involving enamel and dentine without pulp exposure. Periapical and occlusal radiographs would be requested immediately to assess root development, periapical status, and rule out root fracture or alveolar injury.

The candidate would explain to Sarah and her mother in plain language that the 11 displaced palatally we cal lateral luxation(not responding to coold test as the nerve could be in shock or it could be dead this need to be reviewed) . For tooth 21, they would place a glass ionomer or composite resin restoration to protect the exposed dentine. They would decline the request for antibiotics, explaining clearly that antibiotics are not indicated for dental trauma in a healthy child with no signs of infection, and that the pulp treatment itself is the appropriate management.

Post-operative instructions would include soft diet, ibuprofen and paracetamol as per body weight for analgesia, gentle oral hygiene, and an emergency contact number. A follow-up schedule would be arranged at 2 weeks to remove splint , 1 month 2 months 3,6 12months yearly for upto 5 years, monitoring for pulp necrosis, root resorption, and periapical pathology.

Mouthguard fabrication would be strongly recommended before Julie returns to sport.

PQ-15 Five-Year-Old with a Draining Sinus

Paediatric dentistry · 11 minutes · Cluster 1

> ***The station.*** *Child 5 years old girl has mild discomfort on 64/65 which was filled with composite/GIC by school dental van nurse few months ago . Mum also noticed sinus above the tooth recently. Gather information and Answer parent's concern, no need to give treatment plan or diagnosis.*

An excellent candidate begins by warmly introducing themselves to both Sarah and Sally. They establish rapport with Sally using age-appropriate language — perhaps commenting on something she is wearing or holding — before directing most clinical questions to Sarah while keeping Sally engaged. The candidate takes a focused history: asking when the 'pimple' first appeared, how long it has been present, whether it has ever discharged, and whether sally has had any associated pain, fever, facial swelling, or difficulty eating or sleeping. They elicit the history of a toothache approximately four months ago that resolved spontaneously — a critical detail suggesting pulp necrosis. The candidate asks about medical history, allergies, current medications, and immunisation status, confirming sally is fit and healthy with no contraindications to dental treatment. They then address Sarah's concern about diet and caries risk in a non-judgmental way, explaining that early childhood caries is multifactorial — frequency of fermentable carbohydrate exposure , oral hygiene habits, saliva, and tooth morphology all contribute — and that cordial at kindergarten, even if perceived as low-sugar, can be a significant risk factor. The candidate explains that the 'pimple' is likely a sinus tract, a small channel the body has created to drain infection from the tip of the tooth root, and that this indicates the nerve inside the tooth has been affected. They clarify in plain language that a draining sinus actually means the infection is currently releasing pressure, which is why sally is not in acute pain, but that this does not mean the infection has resolved — it requires definitive dental treatment. The candidate reassures Sarah that antibiotics alone would not fix the problem, as the source of infection remains inside the tooth, and that a clinical examination is needed to determine the best course of action. They gain consent for examination using calm, positive language with sally. walk through the management today.

PQ-29 Uncooperative Four-Year-Old Child: Charting and Diet Counselling

Paediatric dentistry · 11 minutes · Cluster 2

The station. *Mia Chen, 4-year-old, first dental visit, presenting with pain. Both Mia and her mother, Mrs Chen, are present today. Bitewing radiographs (attached, left and right — taken despite initially uncooperative behaviour): - Right bitewing: proximal caries into dentine, mesial surface of 84; occlusal caries into dentine, 85. - Left bitewing: proximal caries into dentine, distal surface of 64; early enamel-only proximal caries, mesial of 75. - No periapical pathology or pulpal involvement evident on either film. Diet chart (attached, one typical day as recorded by Mrs Chen): - 7:00am: cereal with milk - 7:30am: apple juice box (#1) - 10:00am (daycare): dried fruit and a muesli bar - 12:00pm…*

An excellent candidate would begin by warmly introducing themselves to both the mother and Mia, crouching to Mia's eye level and using simple, friendly language to reduce anxiety. Recognising this as Frankl Scale 2 (negative) behaviour typical of a first dental visit, the candidate would employ tell-show-do — showing Mia the mirror before touching, using a 'tooth-counting game' framing — alongside positive reinforcement and distraction to gain cooperation. Once Mia allows a brief visual examination, the candidate would systematically chart the primary dentition using Palmer or FDI notation, documenting white spot lesions on 51 and 61, visible plaque on anterior teeth, and correlating bitewing findings showing caries in primary molars. The candidate would then turn to the mother, reviewing the diet chart and asking targeted questions about frequency, type, and timing of sugar and acid intake. They would identify the key early childhood caries (ECC) risk factors: 3–4 apple juice boxes daily providing frequent acid and sugar exposure, a bedtime bottle of milk causing prolonged fermentable carbohydrate contact overnight, and frequent snacking on dried fruit and muesli bars. The candidate would explain these risks clearly and practically — recommending juice be limited to a maximum of 120 mL once daily with meals or eliminated entirely in favour of tap water, that the bedtime bottle be replaced with water only or discontinued, and that snack frequency be reduced with cariogenic snacks swapped for lower-risk alternatives. On oral hygiene, the candidate would address the fluoride toothpaste concern directly, reassuring the mother that a low-fluoride toothpaste (400–550 ppm, such as Colgate Minifluoride) is safe and recommended for children under six, with a pea-sized amount and spitting encouraged. Brushing should occur twice daily, with the parent brushing or supervising until age seven to eight. The candidate would arrange a review appointment in four to six weeks, explain the importance of regular dental visits for monitoring and preventive care, and close with empathy and encouragement.

PQ-34 Management of Dental Trauma in a 3-Year-Old Child

Paediatric dentistry · 11 minutes · Cluster 2

The station. *3-year-old boy comes after 1 week (fell down a week ago) and had a small cut on the lip, mom applied pressure and bleeding stopped. Yesterday she noticed the black discoloration in front tooth (61) and she is here at the dentist. Explain diagnosis, treatment plan and answer questions.*

I would begin by warmly greeting the mother and child, introducing myself and establishing rapport. I would take a structured trauma history: confirming the injury occurred one week ago, the mechanism was a fall from approximately 1.2 metres from a slide platform, witnessed by the mother. I would screen for head injury red flags — asking specifically about loss of consciousness, vomiting, nausea, altered behaviour or drowsiness since the fall, and confirm tetanus immunisation is up to date. I would note the lip laceration was managed with pressure and resolved.

For the examination, I would use a tell-show-do approach, keeping the child calm and involving the mother. I would examine tooth 61 — noting it is an upper left primary central incisor — and identify the black discolouration as consistent with pulp necrosis following intrusion injury sustained one week prior. I would check the occlusion, mobility, and surrounding soft tissues, and note any gingival tenderness or swelling suggesting infection.

I would explain to the mother in plain language: 'The front tooth has been pushed up into the gum from the fall. The dark colour tells us the nerve inside the tooth has been affected. Because this is a baby tooth, we monitor it closely rather than removing it straight away, as it may re-erupt on its own over the next few months.' I would advise a soft diet, age-appropriate analgesia such as paracetamol or ibuprofen, and gentle oral hygiene with a soft toothbrush.

I would counsel the mother on potential sequelae: further discolouration, infection or abscess, ankylosis, and importantly, possible damage to the developing permanent successor tooth including hypoplasia or dilaceration. I would explain that if signs of infection develop — swelling, sinus tract, pain, or failure to re-erupt — extraction would then be indicated.

I would arrange follow-up at one week, six to eight weeks, six months, and yearly until the permanent tooth erupts, with clear safety-net advice to return urgently if swelling, pus, or the child becomes unwell.

PQ-53 Molar Incisor Hypomineralisation in an 8-Year-Old — Sensitivity & Appearance

Paediatric dentistry · 11 minutes · Cluster 2

> ***The station.*** *8-year-old child, attending with mother. Chief Complaint: Mother reports sensitivity in tooth 26 (upper left first permanent molar). She is also concerned about the appearance of the child's front teeth. Examination Findings / Clinical Images: Image 1: Tooth 26 with a well-demarcated yellow-brown opacity and post-eruptive enamel breakdown on the occlusal surface. Image 2: Upper permanent incisors showing creamy-white to yellow opacities with sharp borders against adjacent normal enamel, distributed asymmetrically — 11 and 21 affected, 12 and 22 largely spared. The remaining erupted permanent teeth are unaffected and the primary dentition is sound. Task: Explain the diagnosis to the…*

An excellent candidate begins by warmly greeting both Jenny and Sophie, addressing Sophie directly with an age-appropriate comment to establish rapport and signal that Sophie is the patient. The candidate then takes a structured history, asking Jenny about Sophie's early childhood health — specifically recurrent illnesses and antibiotic use in the first three years of life — eliciting the history of four to five ear infections treated with amoxicillin. The candidate also asks about pregnancy history, eliciting the second-trimester antibiotic course, and importantly does not over-attribute causality to it, recognising that this exposure falls outside the mineralisation window for the affected teeth. Crucially, the candidate asks about family history of similar-looking teeth in parents or siblings — eliciting a clear negative — and takes a fluoride history, establishing optimally fluoridated water, age-appropriate toothpaste and no supplements.

The candidate then explains a single diagnosis: Molar Incisor Hypomineralisation, affecting tooth 26 and the upper central incisors. They explain in accessible language that the enamel on these particular teeth did not form fully while it was developing in Sophie's first few years, that this is linked to the period of recurrent infections, and that only these teeth are affected because only these teeth were forming at that time. The candidate demonstrates diagnostic reasoning by explicitly considering and excluding the two principal differentials: Amelogenesis Imperfecta is ruled out because it is hereditary and would be expected to affect all the teeth in both the baby and adult sets and to appear in other family members, whereas Sophie's baby teeth and most of her adult teeth are sound and the family history is negative; dental fluorosis is ruled out because it produces diffuse, symmetrical mottling across all teeth developing at the same time rather than the sharply demarcated, asymmetric patches seen here, and because the fluoride history is unremarkable. These exclusions are explained to Jenny in lay terms rather than recited as a list.

The candidate explicitly addresses Jenny's guilt, explaining that this condition is not caused by anything she did or did not do during pregnancy or afterwards, and that the childhood ear infections were beyond anyone's control. The emotional concern is validated before reassurance is offered.

For tooth 26, the candidate recommends fluoride varnish, a protective restoration using glass ionomer or composite resin given the post-eruptive breakdown and sensitivity, desensitising advice, and three-monthly review given elevated caries risk, noting that MIH-affected molars can be harder to anaesthetise and need long-term monitoring. For the incisors, the candidate explains that definitive cosmetic treatment such as microabrasion, resin infiltration or composite veneers is deferred until Sophie is around sixteen to eighteen years old when facial growth is complete and the gum margins are stable, with interim fluoride varnish and avoidance of abrasive toothpastes, and offers genuine reassurance that the appearance can be properly addressed in time. Sophie is addressed directly at least once during the management discussion.

PQ-59 MIH in an 11-Year-Old — Second Opinion on GA Extraction

Paediatric dentistry · 11 minutes · Cluster 2

The station. *11-year-old boy, attending with his parents. Chief Complaint: Sensitivity to cold in his back teeth. History: Mother took him to a public hospital previously. General dentist at the hospital suggested extracting the back teeth under general anaesthesia. Parents seeking a second opinion. Examination Findings / Clinical Image: Photograph showing hypomineralised (chalky/opaque) first permanent molars and incisors. Baby teeth appear normal. Diagnosis: Molar Incisor Hypomineralisation (MIH) — affects permanent molars and incisors, primary dentition unaffected. Task: Diagnose and manage. Address parent's concern about the hospital dentist's recommendation for GA extraction.*

An excellent candidate begins by warmly greeting Liam directly — not just his parents — and explains the purpose of the visit in simple terms. They take a focused history, asking Liam whether the sensitivity is triggered by cold or occurs on its own, and elicit that there is no spontaneous or nocturnal pain. They ask Wei about early childhood illnesses and acknowledge the hospitalisation for bronchiolitis at eight months as a plausible aetiological event during the critical window of first permanent molar mineralisation. They reassure both parents clearly and empathetically that MIH is a developmental condition — not caused by diet, sugar, brushing habits, or anything done during pregnancy — and that this reassurance is delivered as a genuine emotional acknowledgement, not a throwaway line. The candidate names the diagnosis as Molar Incisor Hypomineralisation, explains in plain language that the enamel did not harden properly during early childhood, distinguishes it from decay (no bacterial cause), fluorosis (no excess fluoride), and amelogenesis imperfecta (not hereditary, primary teeth unaffected — as seen in the photograph). They present a staged conservative plan: high-fluoride toothpaste and desensitising agents first, fissure sealing of at-risk surfaces, GIC or composite restorations for any areas of surface breakdown, and Hall-technique preformed metal crowns for significantly broken-down molars. They explain that extraction under GA is reserved for teeth with irreversible pulpal involvement or that are unrestorable — neither of which applies to Liam today — and frame the hospital dentist's recommendation diplomatically, acknowledging resource constraints without criticism. They address David's questions about cost and prognosis honestly. They set a three-to-six-month recall and explain that MIH teeth can deteriorate, so monitoring is essential. Throughout, they speak to Liam in age-appropriate terms and acknowledge his embarrassment about his front teeth.

PQ-60 Caries Risk Assessment in a Cooperative 8-Year-Old

Paediatric dentistry · 11 minutes · Cluster 2

> ***The station.*** *Mehmet Askoff, an 8-year-old boy, attends today with his father. His father is worried about black holes in Mehmet's teeth. On examination you find multiple grossly carious primary molars — an intraoral photo is provided, and a bitewing radiograph may sometimes be given. Mehmet is a cooperative child. Conduct a consultation with the father covering the caries risk assessment and management.*

Opening and rapport Greet Mehmet directly first ("Hi Mehmet, I'm just going to have a look at your teeth in a minute — you're going to do a great job") before turning to his father for the history — this signals to the examiner that the child is included, not just discussed over his head. Acknowledge the father's worry without minimising it: "Thanks for bringing him in — let's work out together what's going on and what we can do about it."

Structured history Take a focused caries-risk history rather than a generic diet question:

- Drinks: what does he drink through the day, and how often — plain water, juice, cordial, milk? Sipped throughout the day or with meals only?
- Snacks: what and how often between meals — the frequency matters more than the total amount of sugar.
- Brushing: how often, what toothpaste, supervised or independent, any spots missed or forgotten.
- Fluoride exposure: toothpaste strength, and — a commonly missed question — what's the home water source (town supply, tank, bore)? Non-fluoridated water is a real, addressable risk factor.
- Dental history: when was the last check-up, any previous fillings or extractions, any pain or sleep disturbance now.

In this case: frequent juice/cordial, afternoon snacking on lollies/chips, once-daily unsupervised brushing, non-fluoridated bore water, and an 18-month gap since the last dental visit — a cluster of modifiable risk factors, not one single cause.

Explaining the diagnosis Use the photo (and bitewing, if taken) to show the father what's happening: "These dark areas are decay — the bacteria in the mouth are feeding on the sugars from food and drink and producing acid, which breaks down the tooth." Make the mechanism concrete and non-judgemental: it's the *frequency* of sugar/acid exposure through the day that overwhelms the mouth's ability to repair itself between attacks, not simply "too much sugar" in a moral sense. Explicitly reassure the father this is common and very fixable, not a reflection of bad parenting.

Answering "why did this happen when I thought we were doing okay?" Name the specific contributing factors identified in the history (frequent sugary drinks, afternoon snacking, once-daily brushing, non-fluoridated water, the gap since the last check-up) as a combination, and frame the consult as identifying what to adjust rather than assigning blame.

Answering "can we just watch them since they're baby teeth?" This is the key inflection point of the station. Explain clearly why untreated cavitated primary molars still need active management: they can progress to pulpal involvement causing pain and abscess, a dental abscess in a young child is a genuine medical event (swelling, missed school, possible antibiotics or emergency extraction), premature loss of a primary molar can cause space loss and crowding for the permanent successor, and ongoing pain affects eating, nutrition, and sleep. "Baby teeth hold the space and the bite for years yet — some of these won't be replaced until he's 10 or 11 — so treating decay in them now protects both his comfort today and his adult teeth later."

Management plan

- Restorative: given Mehmet is cooperative, most of the cavitated primary molars can be restored under local anaesthetic with glass ionomer cement or composite; more extensively broken-down teeth may be better managed with a stainless steel crown (e.g. Hall technique or conventional prep) rather than a large filling prone to failure.
- Preventive: switch to plain water as the default drink, confine sugary drinks/snacks to mealtimes rather than grazing through the day, standard-strength fluoride toothpaste (1000-1450 ppm suitable at age 8) twice daily with supervision/checking, in-chair fluoride varnish today and at each recall, and fissure sealants for permanent first molars as they erupt.
- Follow-up: 3-monthly recall given the high caries risk identified today, stepping down to standard intervals once risk factors are addressed and no new lesions are appearing.

Answering "how do we protect his other/adult teeth?" Tie the prevention plan directly back to this question: reducing exposure frequency, fluoride at the right strength and frequency, sealants on permanent molars as they come through, and closer recall until the risk factors are under control.

Closing Summarise the plan in plain terms, check the father's understanding, and give him one or two concrete, realistic first steps to start with (e.g. "swap the after-school snack for something like cheese or fruit, and keep sugary drinks to mealtimes only") rather than an overwhelming list all at once.

Orthodontics

2 stations

PQ-21 Missing Permanent Lateral Incisor

Orthodontics · 11 minutes · Cluster 1

The station. *7-year-old Olivia Smith attends with her mother. Her primary (baby) lateral incisor fell out about a year ago and the permanent lateral has not erupted yet. Her parents are concerned about the gap in the middle and the space between the central incisor and canine. An intraoral photo shows a large midline diastema. photo shows a deep bite Find out more about her situation and answer the parents' queries — explain the reasons for this and the necessary investigations.*

An excellent candidate would begin by warmly greeting Mrs Smith and Olivia establishing rapport before taking a focused history. They would confirm the timeline of the primary lateral incisor exfoliation (approximately one year ago), ask whether any other baby or adult teeth have been slow to erupt or missing, and specifically enquire about a family history of missing or small teeth — noting that hypodontia has a strong genetic component. They would ask about olivia's general health, her dental history.

On examination, the candidate would assess olivias facial profile and skeletal relationship (Class 2), lip competence, and arch form. They would note the spacing in the upper arch, assess the mobility of the retained upper left primary lateral incisor (62), and examine the occlusion.

The candidate would explain the diagnosis and reasons fro diastema to Mrs Smith in plain language

possibilities for the lateral incisor at this age

explains the investigations to be carried out extraoral intraoral - palpate / check for bulge/ frenum attachment/ perio xray phots study scans or models

since they are worrried about xray causing organ damage explaining that normally we recommena occlusal xray ain this situation but since they have concerns about her organ occlusal xray is directed towrds her organ we can manage with 2 IOPAs but that will require 2 xray instead of 1 (autonomy)

explains that the spaces of 1 to 2 mm is normally there at this age anything more than that is less likely to close on its own requiring referral. The candidate would acknowledge the mother's concerns about cost and appearance empathetically and outline a clear referral plan.

explains when asked what will the specialist do? early interceptive orthodontics and opg if we find those teeth are missing on small xray.

ends with review and health promotion supervised brushingand flossing) referral letter to orthodontist. future ortho treatment will put her at higher risk of decay

PQ-44 Assessment and Management of a Missing Maxillary Canine

Orthodontics · 11 minutes · Cluster 1

The station. *Julie, 10-year-old girl, attending with her mother. Chief Complaint / Presentation: Bilaterally missing canines in both jaws. Mum is worried about the child's appearance because of splayed front teeth. The front teeth are described as 'too big.' Eye teeth are missing from both jaws. Parent can feel a bulge in the lower jaw assuming teeth are erupting. There is no bulge in the upper jaw. Clinical Image: Intraoral photo of a child's dentition showing marked spacing/splaying of the front teeth, with missing canines, and prominent gingival/bony bulge in the lower jaw. Task: Gather information and explain what to do.*

An excellent candidate would begin by warmly greeting Julie (Chloe) and her mother Sarah, confirming the child's age and establishing rapport. They would take a focused history covering the timeline of tooth eruption — specifically when the upper left canine erupted, whether any trauma has occurred, any pain or swelling, and family history of similar problems. They would ask about habits such as thumb-sucking and confirm medical history, medications, and allergies.

On examination, the candidate would describe palpating both the buccal and palatal sulci in the upper arch to localise the impacted canine, assess the mobility of the retained deciduous canine (53), evaluate arch space and crowding, and check the occlusion. They would note the absence of a buccal bulge in the upper jaw — a key clinical sign suggesting palatal impaction.

The candidate would request an OPG and correctly interpret it: identifying the upper right permanent canine (13) as palatally impacted, lying horizontally at the level of the apices of 12 and 14 with the crown pointing mesially, and confirming no root resorption of adjacent teeth and no associated pathology. They would note that 23 has already erupted normally.

Explaining findings to Sarah in plain language, the candidate would say the adult eye tooth is stuck behind the roof of the mouth and will not come through on its own. They would explain that early treatment — ideally before age 13 — gives the best chance of success. Treatment involves removing the baby tooth, referring to an orthodontist and oral surgeon for surgical exposure and bonding of a small bracket to guide the tooth into position over approximately 18–24 months. Risks of no treatment include damage to the roots of neighbouring teeth, cyst formation, and ongoing spacing concerns. The candidate would acknowledge Sarah's concerns about cost and appearance, note that private health insurance with orthodontic cover may offset costs, and arrange prompt referral.

Oral medicine

6 stations

PQ-20 Ulcer on lateral border of tongue

Oral medicine · 11 minutes · Cluster 1

The station. *Mrs Cooper 52 year old school teacher, attending today complaining of Ulcer on right side of tongue, present for 2 months her last dental visit was 2 years back. She is fit and healthy. you havent done any examination yet. gather information and Explain the differential diagnosis, manage patient's concern, discuss further investigations needed.*

An excellent candidate would begin by warmly greeting Mrs Cooper and establishing rapport before taking a focused history. They would clarify the duration of the ulcer (5 weeks),and ask specifically about pain, bleeding, and any change in sensation. They would systematically screen for red-flag symptoms including dysphagia, voice change, unexplained weight loss, and night sweats.

In a non-judgmental manner, they would elicit a detailed tobacco history (15–20 cigarettes daily for 45 years, redudeced recently due to cough) and alcohol history (2 standard drinks most evenings, exceeding Australian guidelines), acknowledging these as important contextual factors without shaming the patient. They would note the absence of previous oral ulcers and the family history of lung cancer in a sibling.

examination, they would describe a 1 cm × 1 cm ulcer on the right lateral border of the tongue (middle third) with local trauma source. They would note the patient's comment that his denture feels tight complete examination explained including extraoral / intraolar/ picture / xray of the tooth with disloged filling and its management as per xray. referrals OMDR biopsy/ GP referrral for chest Xray to assess cough and blood tests.

Bilateral cervical lymph node palpation would. The candidate would recognise that the combination of duration exceeding three weeks, lateral border of tongue and cough present. They would communicate this concern to Mrs Cooper with empathy and clarity, avoiding false reassurance, and explain that an urgent referral for incisional biopsy is required — ideally within two weeks in accordance with Australian Head and Neck Cancer guidelines. They would not adopt a watch-and-wait approach.

PQ-26 Assessment and Management of Palatal Swelling

Oral medicine · 11 minutes · Cluster 1

The station. *Mr Michael Jones comes for check-up, he is new to your practice. He is seeing you today and his last check-up was 4 years ago. Large Bluish/dark red swelling on the right side of the palate between 3-7 for 5 years. Complaints about his previous dentist who did RCT filling 4 year ago. How would you gain more information and what special investigation would you do and based on the findings? Give differential diagnosis.*

An excellent candidate would begin by warmly introducing themselves and establishing rapport with Mr Jones, acknowledging his anxiety about the swelling. They would take a structured history of the presenting complaint, noting the swelling has been present for five years on the right palate, asking about onset, rate of growth, any change in size, associated pain, bleeding, discharge, or functional symptoms such as difficulty swallowing, speech changes, or nasal symptoms. They would explore the patient's concern about his previous dentist and the root canal treatment performed four years ago, clarifying which tooth was treated and whether any symptoms followed. Relevant medical history would include smoking status, alcohol use, any previous malignancy, current medications, and allergies. Systemic symptoms including fever, weight loss, and night sweats would be specifically excluded. On examination, the candidate would systematically palpate the swelling, assessing size (approximately 2 cm), consistency (firm), surface texture (smooth), tenderness, and fluctuance. They would examine adjacent teeth 25, 26, and 27 for vitality using an electric pulp tester or cold test, assess for tenderness to percussion, and inspect for any sinus tract. Cervical lymph nodes would be palpated bilaterally. Based on findings — a firm, smooth, non-tender, fluctuant 2 cm palatal swelling with vital adjacent teeth and no lymphadenopathy — the candidate would formulate a differential diagnosis led by a minor salivary gland neoplasm, most likely pleomorphic adenoma, with malignant salivary gland tumour, palatal abscess, and nasopalatine duct cyst as alternatives. They would explain the need for CT or MRI imaging and urgent referral to an oral and maxillofacial surgeon or oral medicine specialist within two weeks for incisional biopsy. The candidate would sensitively address the patient's cancer concerns, provide safety-netting advice regarding rapid growth, pain, or dysphagia, and document findings thoroughly.

PQ-33 Assessment and Management of Gingival Ulceration

Oral medicine · 11 minutes · Cluster 1

The station. *Mr. Smith is a 65-year-old patient, attending your surgery for the first time today asking for a lower denture to replace his missing teeth. You examined the patient, and you find that the patient is wearing an upper complete denture for the past 8 years with no lower denture. He also complains of angular cheilitis on the right side of his lips. You also notice an ulcer; the patient says that the ulcer has been present there for the last 2 months since he had his lower canine removed. The ulcer is about 2-3mm in size, it's not bleeding, and patient does not experience any pain. He works as a miner and is leaving for work after 3 months. He won't be back until 3 months. How will you…*

I would begin by introducing myself and establishing rapport with Mr. Smith, acknowledging his concern about the ulcer given his family history of oral cancer. I would take a focused history covering the duration of the ulcer (2 months), the precipitating trauma (lower canine extraction), pain characteristics (currently non-tender per patient report, though tender on examination), and whether he has had similar ulcers before. I would ask specifically about red-flag symptoms including difficulty swallowing, unexplained weight loss, hoarseness, and any other oral lesions. I would explore his social history including smoking and alcohol use, and note his occupation as a miner for potential chemical or dust exposure. I would ask about systemic symptoms such as fever, skin or genital lesions, gastrointestinal symptoms, eye problems, and joint pain to exclude conditions such as Behçet's disease, Crohn's disease, or reactive arthritis. On examination, I would systematically assess the ulcer: site (attached gingiva adjacent to 36), size (1.5 cm), shape (irregular), base (yellowish-white), borders (raised and rolled), and surrounding erythema. I would gently palpate the ulcer for induration and examine all cervical and submandibular lymph node chains for lymphadenopathy. My differential diagnosis would include traumatic ulcer as the most likely given the extraction history, but the 2-month duration, irregular shape, raised rolled borders, and size of 1.5 cm raise significant concern for malignancy and mandate urgent investigation. Aphthous ulceration is less likely given the single episode, size, and border characteristics. Given the ulcer has been present for more than 3 weeks and has suspicious features, I would not adopt a watch-and-wait approach. I would refer urgently for incisional biopsy, ideally within 2 weeks, and explain this clearly and sensitively to Mr. Smith. I would advise chlorhexidine 0.2% mouthwash, avoid trauma to the area, and arrange urgent specialist referral before he leaves for work in 3 months. I would document all findings thoroughly.

PQ-43 Management of Recurrent Aphthous Ulceration

Oral medicine · 11 minutes · Cluster 1

The station. *Charlotte, 20-year-old student. Chief Complaint: Ulceration for the past 3 days. She is a law student in a lot of stress because of her final exam starting in 2 months. Her sister gets similar ulcers as well. Examination Findings: Ulcer found on the inner side of her lips. Clinical Image: Intraoral photo showing an ulcer on the inner mucosal surface of the lip — consistent with aphthous ulceration. Task: Explain what special test you will do and give your differential diagnosis along with your provisional.*

I would begin by introducing myself to Charlotte and establishing rapport, acknowledging the discomfort she is experiencing. I would take a focused history exploring the frequency, duration, site, size, and number of ulcers, noting she reports episodes every 8–12 weeks lasting 7–10 days, with the current ulcer present for 3 days. I would ask about potential triggers including stress, dietary factors, trauma, new toothpaste or oral care products, and whether episodes correlate with her menstrual cycle. I would enquire about family history of similar ulcers, which is relevant given her sister is affected. To exclude systemic associations, I would ask about gastrointestinal symptoms such as diarrhoea, bloating, or weight loss to screen for coeliac disease or inflammatory bowel disease, as well as genital ulcers, ocular symptoms such as uveitis, and skin lesions to exclude Behçet's syndrome. I would confirm she is not on any regular medications and has no known medical conditions. On examination, I would describe the ulcer as approximately 5 mm in diameter, round, shallow, with a yellow-grey fibrinous base and erythematous halo on the labial mucosa, with no other lesions present. My provisional diagnosis is recurrent minor aphthous ulceration, a benign condition likely triggered by stress and possibly a genetic predisposition given the family history. For special investigations, I would request blood tests including full blood examination, iron studies, serum B12, folate, and coeliac serology to exclude haematinic deficiencies and coeliac disease as underlying contributors. Management would include topical corticosteroids such as triamcinolone acetonide paste, benzydamine mouthwash for symptomatic pain relief, and a protective barrier gel. I would advise stress management strategies, avoidance of known trigger foods, and good oral hygiene. I would reassure Charlotte that this is a benign condition, but advise her to return if any ulcer persists beyond three weeks, increases in size, or if systemic symptoms develop, as further investigation or specialist referral would then be warranted.

PQ-50 Incidental White Lesion — Differential Diagnosis & Biopsy Decision

Oral medicine · 11 minutes · Cluster 1

The station. *Hanna, 45-year-old, new patient. Attending for the first time requesting a routine check-up and clean. Incidental Finding: On intraoral examination, you notice a white patch on the right side of the mouth (buccal mucosa / lateral tongue region adjacent to an amalgam restoration on the right side). Patient is not aware of it. History: Smokes occasionally. Currently under a lot of stress. No other significant medical history mentioned. Clinical Image: Intraoral photo showing a white lesion on the buccal mucosa/lateral border — requires differential diagnosis. Task: Gather information. Determine what special investigations are required. Explain findings and provide a differential diagnosis.*

An excellent candidate begins by systematically describing the incidental white lesion before initiating history-taking. They note the site (right buccal mucosa adjacent to the amalgam restoration), approximate size, colour (homogeneous white), surface texture (slightly raised or flat), border definition, and critically confirm that the lesion cannot be wiped off with gauze — distinguishing it from pseudomembranous candidiasis. The candidate then takes a targeted history, sensitively informing Hanna of the finding without alarming her. They elicit her smoking history accurately — not accepting 'social smoker' at face value but quantifying it as five to ten cigarettes per day over more than twenty years — and ask about alcohol consumption, revealing approximately six to eight standard drinks per week. They ask specifically about previous oral changes or white patches, uncovering the prior episode on the left buccal mucosa two years ago that resolved spontaneously — a critical red flag for field cancerisation. They confirm the absence of pain, bleeding, dysphagia, or difficulty eating, and note the proximity of the lesion to the amalgam restoration as a potential aetiological factor. The differential diagnosis is presented in a clinically reasoned order: lichenoid reaction to the adjacent amalgam is the most anatomically plausible; frictional keratosis from cheek biting under stress is reasonable; leukoplakia is the diagnosis of exclusion; and oral squamous cell carcinoma must be named as a condition to exclude, framed carefully and without catastrophising. The management plan includes immediate clinical photography and measurement for baseline documentation, consideration of toluidine blue staining, and urgent referral to an oral medicine specialist or oral and maxillofacial surgeon for biopsy within two weeks. Amalgam replacement is discussed as a supplementary diagnostic manoeuvre, not a substitute for referral. The candidate communicates honestly and compassionately, acknowledging Hanna's anxiety, explaining that most white patches are benign, and reassuring her that prompt investigation is the right and responsible course of action.

NEW Enamel Defects in a Young Adult — Wants Veneers

Oral medicine · 11 minutes · Cluster 2

The station. *Ms Tara Beckett, 24, a graphic designer, has come to your practice unhappy with the appearance of her upper front teeth. She says they have "always been patchy and pitted" and she wants veneers before her sister's wedding in three months. On examination the permanent incisors and first molars show creamy-yellow to brown opacities with horizontal grooving and pitting; the pattern is symmetrical across all four quadrants. There is no caries and her oral hygiene is good. Take a history, explain what you think is going on, and discuss management.*

This station looks cosmetic and is not. Tara has undiagnosed coeliac disease, and her enamel is the visible record of it. The whole station turns on whether the candidate asks why the enamel looks like this before agreeing to cover it with porcelain.

START BY TAKING THE CONCERN SERIOUSLY

She came in about her smile and a family wedding. If the first thing you do is redirect to her bowels, you lose her. Something like: "I can see why you're bothered by them, and I do think we can improve how they look. Before we talk about how, can I ask a bit about the teeth themselves and about your general health?

The way they've formed tells us something, and it'll change what I'd recommend."

READING THE ENAMEL

The finding is not "staining" — it is a pattern. Creamy-yellow to brown opacities with horizontal grooving and pitting, affecting the permanent incisors and first molars, SYMMETRICAL across all four quadrants and matching left to right. Canines and premolars are largely spared.

That symmetry is the diagnostic feature, and it is what separates this from the usual suspects:

- Dental fluorosis is diffuse and generalised, not confined to tooth groups in an ordered way.
- Molar-incisor hypomineralisation is characteristically patchy and ASYMMETRIC — one molar badly affected, its opposite number fine.
- Amelogenesis imperfecta affects the whole dentition and runs in families; here the family's teeth are normal.
- Tetracycline staining is banded and has a drug history behind it.

Symmetrical and chronological means the insult was systemic and time-limited — it hit whatever was mineralising at the time, in every quadrant at once. In a young adult, that points at early childhood illness or malabsorption, and coeliac disease belongs high on that list.

THE HISTORY THAT UNLOCKS IT

None of this is volunteered. Ask specifically:

- Have they always looked like this, or did they change? (Always — they came through like it.)
- Fluoride growing up — fluoridated water, supplements, swallowing toothpaste? (Non-fluoridated area, no supplements. Argues against fluorosis.)
- Any long illness or medication as a small child?
- Do you get mouth ulcers? (Constantly, three or four at a time, for as long as she can remember, healing in about a week — minor aphthous.)
- How's your general health? Ever been told you're anaemic? (Twice. Iron tablets "didn't really do much" — that is the detail that matters: iron deficiency that does not correct is a malabsorption flag.)
- Any tummy trouble — bloating, bowel habit? (Bloating and loose stools for years, filed under "just IBS", never investigated. She will play it down; ask again.)
- Does anything run in the family? (Her older sister was diagnosed coeliac two years ago. This is the single most unlocking question in the station.)

Your own Therapeutic Guidelines corpus makes the ulcer link explicitly: coeliac disease is listed as one of the conditions to investigate in recurrent aphthous ulceration, alongside iron/B12/folate/zinc deficiency, ulcerative colitis, Behçet syndrome and PFAPA (TG Oral and Dental, p161).

PUTTING IT TO HER

Plain language, no jargon: "Your teeth formed like this when you were small — the enamel was being laid down and something interfered with it. On its own that could be a few things. But put together with the ulcers you've always had, being anaemic twice and the iron not really fixing it, the tummy symptoms, and your sister having coeliac — I think it's worth getting you tested for coeliac disease. Your teeth may have been the first sign of it."

THE REFERRAL

GP, for coeliac serology — tissue transglutaminase IgA, with a total IgA (because selective IgA deficiency gives a false negative). Confirmation is medical, usually a duodenal biopsy in adults, and is the GP's or gastroenterologist's call. Spotting it and routing it is the dentist's job; diagnosing it is not.

THE SAFETY POINT — THE MOST IMPORTANT THING YOU SAY

She will tell you she is planning to go gluten free next week because a friend felt better on it. Stop that:

"Please don't start yet — and this is the one thing I really want you to take away. The test for coeliac disease only works while you're still eating gluten. If you cut it out first, the blood test and the biopsy can come back normal even if you do have it, and then you're stuck not knowing. Keep eating normally until your GP has tested you. After that, if it's positive, gluten free for life — but let's get the answer first."

This is the trap the station is built around, because the intuitive kind response — "sure, give it a go, can't hurt" — is the harmful one.

BE HONEST ABOUT WHAT DIET WON'T FIX

"Going gluten free will help your gut, your energy and your ulcers. It won't change these teeth. The enamel formed the way it formed and it doesn't repair itself — improving how they look is a dental job, not a dietary one." Teeth that mineralise after diagnosis can form normally; hers are already through.

THE TEETH THEMSELVES

Don't commit to veneers today, and say why — not to withhold treatment, but because sequencing matters and you don't start irreversible work on someone mid-diagnosis. Then be concrete about the options so she doesn't hear a flat no:

- Microabrasion and/or external bleaching for the milder discolouration — least destructive, worth trying first.
- Composite — additive, repairable, cheaper, achievable quickly. A reasonable interim if she wants something before the wedding.
- Ceramic veneers — best aesthetics and longevity, most tooth removal, most cost. Bonding to hypomineralised enamel is less predictable than to sound enamel, which is worth telling her honestly.

Three months is workable for definitive treatment. Say so, but don't over-promise a result that depends on how she responds to the earlier steps.

COMMON WAYS TO FAIL THIS STATION

Discussing veneer shades for ten minutes and never asking why the enamel is defective. Calling it "staining". Listing only fluorosis and MIH and never reaching for a systemic cause. Never asking about family history. Agreeing that gluten free sounds like a good idea, or letting it pass unchallenged. Telling her a gluten-free diet will improve her teeth. Refusing treatment outright without offering a pathway, so she leaves feeling dismissed about the thing she actually came in for.

Medical emergency

2 stations

PQ-28 Management of Facial Cellulitis in an Uncontrolled Diabetic Patient

Medical emergency · 11 minutes · Cluster 2

The station. *Mr James Oliver is a new patient in your clinic, complaining of severe pain and facial swelling related to his upper right side for some time. Examination reveals that the upper right first molar is badly carious and beyond restoration, and now it is indicated for removal. He went to see GP before; he gave him antibiotics but it didn't seem to work so GP referred him to see you. Swelling is still there, and he is in a lot of pain. His medical record shows that he has diabetes for the last 8 years and for which he is using metformin. How would you plan his management and gain consent for his treatment plan and discuss possible complications that may happen?*

I would begin by introducing myself and establishing rapport with Mr Oliver, then take a focused history covering the onset and progression of his swelling, any systemic symptoms such as fever, malaise, difficulty swallowing, changes in voice, or shortness of breath — all of which would indicate airway compromise requiring immediate escalation. I would elicit a detailed diabetes history including his current medications, compliance, last HbA1c, last blood glucose reading, and time of last meal and drink, as these directly affect anaesthetic and surgical risk. I would then perform a systematic examination: recording vital signs including temperature, pulse, blood pressure, respiratory rate and oxygen saturation; assessing the extent, warmth, erythema and fluctuance of the swelling; measuring inter-incisal opening to quantify trismus; and evaluating the airway. Intra-orally, I would identify the likely odontogenic source — the grossly carious upper right first molar — and assess for vestibular swelling or pus. Given the combination of spreading facial cellulitis, pyrexia of 38.4°C, trismus, poorly controlled diabetes with an HbA1c of 9.2%, and failure to respond to oral antibiotics, I would recognise this as a medical emergency and arrange immediate hospital admission. I would check a point-of-care blood glucose level immediately and arrange urgent bloods including FBC, CRP, U&Es, and blood cultures. I would contact the on-call medical team to co-manage his diabetes and initiate IV fluid resuscitation. Antibiotic therapy would be escalated to IV amoxicillin-clavulanate or amoxicillin plus metronidazole, with clindamycin as an alternative if penicillin allergy were present. Imaging — OPG and CT neck with contrast if deep space involvement is suspected — would be arranged. Definitive surgical management, comprising extraction of the causative tooth and incision and drainage if fluctuance is present, would be performed once the patient is medically stabilised and blood glucose is optimised, ideally in a hospital operating theatre under appropriate anaesthetic cover. I would obtain informed consent explaining the diagnosis, proposed treatment, and risks including airway compromise, spread to deep neck spaces, sepsis, and anaesthetic risks related to his diabetes.

PQ-31 Management of Cellulitis in an Asthmatic Patient

Medical emergency · 11 minutes · Cluster 3

The station. *Mr Thomas Edison is attending your surgery today, complaining of pain and swelling associated with one of his teeth on the top left side. Upon examination, you find that he has facial swelling, swollen lymph nodes and has a fever. Xray shows deep caries related to that tooth, with complex root morphology, roots of the root look in close approximation to the maxillary sinus. Patient has an appointment with the oral surgeon tomorrow morning for the removal of this tooth. Medical history given, allergy to penicillin and has Asthma. Manage patients' concerns and give an appropriate treatment plan.*

An excellent candidate would begin by warmly greeting Mr Edison and establishing rapport before taking a focused history. They would clarify the nature of his penicillin allergy — specifically that he developed an isolated rash as a child, with no history of anaphylaxis, urticaria, angioedema, or bronchospasm — recognising this as a low-risk, non-immediate reaction rather than true IgE-mediated hypersensitivity, which changes the safe alternative options available. They would then establish medication compliance, identifying that he stopped amoxicillin after two doses three days ago due to recalling his allergy, and that no alternative antibiotic was sought. The candidate would assess his asthma status, confirming he uses Seretide twice daily and Ventolin as required, noting increased reliever use yesterday (twice) as a sign of suboptimal control. A thorough clinical examination would follow: recording temperature (38.2°C), assessing the extent of facial swelling, checking for trismus, testing ability to swallow, and evaluating for signs of airway compromise. The candidate would recognise this as spreading odontogenic cellulitis with systemic involvement — fever, indurated erythematous swelling, lymphadenopathy, and dysphagia — representing a high-risk scenario requiring urgent hospital admission rather than outpatient management. They would explain clearly to the patient that the infection has spread beyond the tooth and that oral antibiotics alone are insufficient at this stage. The candidate would plan IV antibiotic therapy, selecting a cephalosporin (e.g. ceftriaxone IV, or cephalexin orally) as the appropriate alternative — since cross-reactivity between penicillins and cephalosporins is low and his reaction history is a mild, non-immediate rash rather than anaphylaxis — reserving clindamycin for patients with a genuine immediate/severe penicillin hypersensitivity, and noting that macrolides carry interaction risks with his inhaled corticosteroid. They would arrange urgent hospital referral for IV antibiotics, possible incision and drainage, and definitive extraction of tooth 36, coordinating with the oral surgeon already scheduled for the following morning. Perioperative asthma management would be addressed — ensuring the patient has her Ventolin inhaler with her, confirming asthma is as well-controlled as possible before any procedure, and flagging her asthma status to the treating hospital team. The candidate would acknowledge the patient's concerns about work and childcare with empathy while clearly communicating the clinical urgency.

Communication & ethics

3 stations

PQ-18 Dietary Advice for an Athletic Patient

Communication & ethics · 11 minutes · Cluster 3

The station. *Mr Sam Smith is a 28-year-old gym trainer. You have already done the examination in the last appointment and noticed several interproximal caries and erosion. You also noticed that his saliva is thick and stringy. You gave him oral hygiene instructions, advised to get some restorations and to fill the diet chart. He would like to know how to modify his diet. He is very health conscious, following good diet regime. Diet chart given: He is drinking lots of sports drink, coffee and tea with sugar, cereal bars, fizzy drinks fruits. He is fit and healthy. Answer patients concern and inform what are the changes he requires to implement. No need to give discuss treatment plan for caries.*

An excellent candidate would begin by warmly welcoming Mr Smith and acknowledging his health-conscious lifestyle before reviewing the completed diet chart in a non-judgmental way. They would take a structured dietary history, clarifying the frequency, timing, and type of foods and drinks consumed — noting the sports drinks, flavoured fizzy drinks, coffee and tea with sugar, cereal bars, and fruit. confirming which medications he uses. The candidate would explain, in plain language, that sports drinks and fizzy drinks are both high in sugar and highly acidic, and that sipping them throughout the day — rather than consuming them in one sitting — dramatically increases the time teeth are under acid attack, contributing to both decay and enamel erosion. They would advise switching to plain water as the primary hydration source and limiting sports drinks to during intense exercise only, followed by a plain water rinse. The candidate would highlight that cereal bars and dried fruit, despite their healthy image, are sticky and high in fermentable sugars, and suggest alternatives such as fresh fruit, cheese, unsalted nuts, or yoghurt. They would recommend reducing sugar in tea and coffee, or switching to sugar-free options. The candidate would recommend using fluoride 5000ppm toothpaste twice daily, waiting 30 minutes after eating before brushing, and consider recommending a remineralising agent such as Tooth Mousse given the thick, stringy saliva suggesting reduced salivary flow. They would check Mr Smith's understanding using the teach-back method, acknowledge his motivation, and arrange a review appointment. and recommend meeting a sport nutritionist and reading the labels

recommend review 3monthly and flouride varnish

PQ-36 Antibiotic Prophylaxis in Patient with Joint Replacement

Communication & ethics · 11 minutes · Cluster 3

The station. *Mark Danial is a new patient in your surgery. He is seeing you today, complaining about his upper canine 23 tooth, which is extremely mobile due to bone loss. PA of 23,24,25 was given with 23 severe bone loss (there is bone loss around 24 and 25 as well). 23 roots are close to the anterior wall of the sinus. You decided to extract 23. Patient has joint replacement 6 years ago. He is Allergic to penicillin. He wants antibiotic prophylaxis as his previous dentist was giving him before dental treatment. He is also having asthma and hypertension. Explain the management plan to the pt.*

An excellent candidate would begin by warmly greeting Mark and acknowledging his concern about antibiotic prophylaxis before proceeding to take a focused medical history. They would confirm the type of joint replaced, clarify that it was performed six years ago, ask whether there have been any complications, infections, or revisions since surgery, and screen for immunocompromising conditions including diabetes, steroid use, or immunosuppressive medications. They would note his penicillin allergy and his diagnoses of asthma and hypertension.

The candidate would then explain the planned procedure — extraction of 23, noting the proximity to the maxillary sinus and the need for careful technique — and address the antibiotic prophylaxis question directly and empathetically. They would explain that current Australian guidelines, consistent with the ADA and AAOS, do not recommend routine antibiotic prophylaxis for patients with uncomplicated joint replacements in immunocompetent patients, particularly when the replacement is more than two years old with no history of infection or revision. They would validate Mark's previous experience while gently clarifying that the evidence base has evolved.

Given his penicillin allergy, the candidate would highlight that prescribing antibiotics unnecessarily carries a meaningful risk of adverse drug reaction — approximately 3% — which is actually higher than the estimated risk of a prosthetic joint infection arising from a dental procedure, approximately 0.3%. This risk-benefit reasoning would be communicated clearly and without jargon.

The candidate would acknowledge that prophylaxis may be appropriate in specific circumstances — within two years post-surgery, in immunocompromised patients, or those with a history of prosthetic joint infection — and offer to liaise with Mark's orthopaedic surgeon if he remained concerned. They would confirm his understanding, invite questions, and document the discussion. Throughout, they would maintain a calm, empathetic, and evidence-based approach.

PQ-58 Drug-Seeking Behaviour — Opioid Request for Dental Pain

Communication & ethics · 11 minutes · Cluster 1

> ***The station.*** *Female patient. Tooth known to need RCT — previous dentist diagnosed irreversible pulpitis and recommended root canal treatment. Scenario: Patient presents demanding strong prescription painkillers (specifically fentanyl, oxycodone, or tramadol) for toothache. Claims her regular dentist is away and she cannot contact him. States normal painkillers are not effective. She also has a physiotherapist-prescribed pain relief patch for chronic back pain. Has an appointment with her regular dentist next week. Insisting she needs strong medication urgently. Key Complexity: Drug-seeking behaviour. Task: Manage the patient's request. Address her concerns without prescribing opioids. Provide safe…*

This station tests the candidate's ability to recognise drug-seeking behaviour, maintain professional boundaries, and manage acute dental pain appropriately without prescribing opioids. On presentation, several red flags are immediately apparent: the patient is requesting specific opioids by name (fentanyl, oxycodone, tramadol), claims her regular dentist is unavailable, states that standard analgesics are ineffective, and already has an opioid-adjacent medication (fentanyl patch) prescribed for a separate condition. These cues collectively raise concern for drug-seeking behaviour or opioid misuse. The candidate should acknowledge the patient's pain empathetically and non-judgementally, validating that irreversible pulpitis is genuinely painful. However, the candidate must clearly and calmly decline to prescribe opioids, explaining that opioids are not first-line dental analgesics, carry significant risks including dependence and interaction with her existing patch, and are not indicated when definitive dental treatment is available. Appropriate management includes prescribing or recommending a combination of ibuprofen 400 mg and paracetamol 500–1000 mg alternating every 3–4 hours, which has evidence-based efficacy superior to opioids for dental pain. If infection is suspected, antibiotics such as amoxicillin should be considered. The candidate should offer an urgent dental referral or same-day emergency dental appointment rather than pharmacological substitution. Safety-netting includes advising the patient to present to an emergency department if she develops systemic signs of infection, trismus, or swelling. The refusal to prescribe opioids, the clinical rationale, and the alternative management plan must be clearly documented in the patient record.